



Anthony Dimarcello "Anthony"

Patient Health Summary, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Allergies

No known active allergies

Medications

HYDROcodone-acetaminophen (NORCO) 10-325 mg tablet (Started 9/14/2024)
Take 0.5 tablets by mouth every 6 (six) hours if needed for moderate pain (4-6) for up to 12 doses.

Active Problems

Not on file

Social History

Tobacco Use	Types	Packs/Day	Years Used	Date
<u>Smoking Tobacco: Some Days</u>				
Housing Stability		Answer	Date Recorded	
Housing Stability		Not on file	08/05/2023	
Housing Stability		Not on file	08/05/2023	
Interpersonal Safety		Answer	Date Recorded	
Interpersonal Safety		Not on file	12/01/2024	
Interpersonal Safety		Not on file	12/01/2024	
Interpersonal Safety		Not on file	12/01/2024	
Interpersonal Safety		Not on file	12/01/2024	
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Male	01/22/2025 10:41 AM PST	
Legal Sex		Male	05/31/2023 8:08 PM PDT	
Gender Identity		Male	01/22/2025 10:41 AM PST	
Sexual Orientation		Straight	01/22/2025 10:41 AM PST	

Last Filed Vital Signs

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	<u>132/81</u>	<u>09/14/2024 4:46 PM PDT</u>	
Pulse	<u>91</u>	<u>09/14/2024 4:46 PM PDT</u>	
Temperature	<u>36.8 °C (98.3 °F)</u>	<u>09/14/2024 4:46 PM PDT</u>	
Respiratory Rate	<u>20</u>	<u>09/14/2024 4:46 PM PDT</u>	
Oxygen Saturation	<u>98%</u>	<u>09/14/2024 4:46 PM PDT</u>	
Inhaled Oxygen Concentration	-	-	
Weight	<u>85.7 kg (188 lb 15 oz)</u>	<u>09/14/2024 3:10 PM PDT</u>	
Height	<u>182.9 cm (6')</u>	<u>09/14/2024 3:10 PM PDT</u>	
Body Mass Index	<u>25.62</u>	<u>09/14/2024 3:10 PM PDT</u>	

Results

X-Ray (XR) Left Shoulder 2 or More Views (XR SHOULDER LEFT 2+ VW) - Final result (09/14/2024 4:06 PM PDT)

Anatomical Region	Laterality	Modality
Shoulder	Left	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/14/2024 4:08 PM PDT	

Impressions

09/14/2024 4:10 PM PDT

1. Redemonstrated comminuted fracture of the left humeral neck.

Electronically signed by January Lopez 9/14/2024 4:10 PM

Narrative

09/14/2024 4:10 PM PDT

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT SHOULDER, 4 views

CLINICAL HISTORY: Left shoulder pain after trauma.

COMPARISON: August 29, 2024.

FINDINGS:

There is redemonstration of a comminuted fracture through the left humeral neck with slight lateral displacement of the shaft relative to the head. There is evidence of hemarthrosis with widening of the subacromial space. The acromioclavicular joint is intact.

Procedure Note

Dr. J Lopez, MD - 09/14/2024

Formatting of this note might be different from the original.

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT SHOULDER, 4 views

CLINICAL HISTORY: Left shoulder pain after trauma.

COMPARISON: August 29, 2024.

FINDINGS:

There is redemonstration of a comminuted fracture through the left humeral neck with slight lateral displacement of the shaft relative to the head. There is evidence of hemarthrosis with widening of the subacromial space. The acromioclavicular joint is intact.

IMPRESSION:

1. Redemonstrated comminuted fracture of the left humeral neck.

Electronically signed by January Lopez 9/14/2024 4:10 PM

Authorizing Provider	Result Type	Result Status
C Caputo PA-C	IMG XR PROCEDURES	Final Result

X-Ray (XR) Chest 1 View (XR CHEST 1 VW) - Final result (08/29/2024 9:58 PM PDT)

Anatomical Region	Laterality	Modality
Chest		Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time		
			08/29/2024 10:12 PM PDT			
Impressions						
08/29/2024 10:12 PM PDT						
1. No evidence of acute cardiopulmonary disease.						
Electronically signed by John Lieu 8/29/2024 10:12 PM						
Narrative						
08/29/2024 10:12 PM PDT						
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO						
CHEST, 1 VIEW						
CLINICAL HISTORY: TRAUMA.						
COMPARISON: Chest x-ray 2/11/2024						
FINDINGS:						
The lungs are clear. The heart size is within normal limits.						
No acute osseous process.						
Procedure Note						
Dr. J Lieu, MD - 08/29/2024						
Formatting of this note might be different from the original.						
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO						
CHEST, 1 VIEW						
CLINICAL HISTORY: TRAUMA.						
COMPARISON: Chest x-ray 2/11/2024						
FINDINGS:						
The lungs are clear. The heart size is within normal limits.						
No acute osseous process.						
IMPRESSION:						
1. No evidence of acute cardiopulmonary disease.						
Electronically signed by John Lieu 8/29/2024 10:12 PM						
Authorizing Provider	Result Type	Result Status				
Dr. J Houpt MD	IMG XR PROCEDURES	Final Result				
12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (08/29/2024 8:14 PM PDT)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG	89	bpm			HOAG CV IECG RESULTING	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>ATRIAL RATE</u>	<u>87</u>	<u>bpm</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>P-R INTERVAL</u>	<u>134</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>P WAVE AXIS</u>	<u>76</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>QRS DURATION</u>	<u>97</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Q-T INTERVAL</u>	<u>373</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>455</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>QRS AXIS</u>	<u>56</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>T AXIS</u>	<u>45</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>INTERPRETATION TEXT</u>	<u>- NORMAL</u> <u>ECG -</u> <u>Sinus</u> <u>rhythm</u> <u>when</u> <u>compared</u> <u>with ECG of</u> <u>11-Feb-2024</u> <u>10:07:33,</u> <u>Nonspecific</u> <u>significant</u> <u>change</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>INTERPRETATION TEXT</u>	<u>Electronically</u> <u>signed by:</u> <u>Gandhi,</u> <u>Radhakrishan</u> <u>29-Aug-2024</u> <u>22:10:05</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Specimen (Source)</u>	<u>Anatomical Location /</u> <u>Laterality</u>		<u>Collection Method /</u> <u>Volume</u>		<u>Collection Time</u>	<u>Received Time</u>
					<u>08/29/2024 8:14 PM PDT</u>	
<u>Narrative</u>						
<u>This result has an attachment that is not available.</u>						
<u>-</u>						
<u>Authorizing Provider</u>	<u>Result Type</u>		<u>Result Status</u>			
<u>Dr. J Houpt MD</u>	<u>ECG ORDERABLES</u>		<u>Final Result</u>			
<u>Performing Organization</u>	<u>Address</u>		<u>City/State/ZIP Code</u>		<u>Phone Number</u>	
<u>HOAG CV IECG</u> <u>RESULTING</u>					<u>-</u>	
<u>X-Ray (XR) Left Humerus 2 or More Views (XR HUMERUS LEFT 2+ VW) - Final result (08/29/2024 3:56 AM PDT)</u>						
<u>Anatomical Region</u>		<u>Laterality</u>		<u>Modality</u>		
<u>Upper Extremities</u>		<u>Left</u>		<u>Digital Radiography</u>		

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			08/29/2024 7:47 AM PDT	
Impressions				
08/29/2024 7:48 AM PDT				
1. Acute mildly displaced proximal left humerus fracture.				
Electronically signed by Gerard Nguyen 8/29/2024 7:48 AM				
Narrative				
08/29/2024 7:48 AM PDT				
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO				
LEFT HUMERUS, 2 VIEWS				
CLINICAL HISTORY: Left shoulder pain.				
COMPARISON: None.				
FINDINGS:				
Acute proximal humerus neck fracture with lateral displacement approximately one half shaft width. The joint spaces are preserved. No significant soft tissue swelling.				
Procedure Note				
Dr. G Nguyen, MD - 08/29/2024				
Formatting of this note might be different from the original.				
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO				
LEFT HUMERUS, 2 VIEWS				
CLINICAL HISTORY: Left shoulder pain.				
COMPARISON: None.				
FINDINGS:				
Acute proximal humerus neck fracture with lateral displacement approximately one half shaft width. The joint spaces are preserved. No significant soft tissue swelling.				
IMPRESSION:				
1. Acute mildly displaced proximal left humerus fracture.				
Electronically signed by Gerard Nguyen 8/29/2024 7:48 AM				
Authorizing Provider	Result Type	Result Status		
Dr. E Kang MD	IMG XR PROCEDURES	Final Result		
X-Ray (XR) Left Shoulder 2 or More Views (XR SHOULDER LEFT 2+ VW) - Final result (08/29/2024 3:55 AM PDT)				
Anatomical Region	Laterality	Modality		
Shoulder	Left	Digital Radiography		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			08/29/2024 7:48 AM PDT	

08/29/2024 7:49 AM PDT

1. Acute mildly displaced proximal left humerus fracture.

Electronically signed by Gerard Nguyen 8/29/2024 7:49 AM

Narrative

08/29/2024 7:49 AM PDT

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

XR SHOULDER LEFT 2+ VW

CLINICAL HISTORY: Left shoulder pain.

COMPARISON: None.

FINDINGS:

Acute comminuted proximal humerus neck fracture with lateral displacement approximately one half shaft width. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

Procedure Note

Dr. G Nguyen, MD - 08/29/2024

Formatting of this note might be different from the original.

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

XR SHOULDER LEFT 2+ VW

CLINICAL HISTORY: Left shoulder pain.

COMPARISON: None.

FINDINGS:

Acute comminuted proximal humerus neck fracture with lateral displacement approximately one half shaft width. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

IMPRESSION:

1. Acute mildly displaced proximal left humerus fracture.

Electronically signed by Gerard Nguyen 8/29/2024 7:49 AM

Authorizing Provider	Result Type	Result Status
Dr. E Kang MD	IMG XR PROCEDURES	Final Result

TROPONIN I - Final result (02/11/2024 10:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	<0.01	0.00 - 0.03 ng/mL		02/11/2024 10:58 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time		
<u>Blood</u>	<u>Venous blood specimen / Unknown</u>	<u>Venipuncture / Unknown</u>	<u>02/11/2024 10:19 AM PST</u>	<u>02/11/2024 10:24 AM PST</u>		
Narrative						
—						
Authorizing Provider	Result Type	Result Status				
<u>Dr. H Marino DO</u>	<u>LAB BLOOD ORDERABLES</u>	<u>Final Result</u>				
Performing Organization	Address	City/State/ZIP Code	Phone Number			
<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	<u>One Hoag Drive</u>	<u>Newport Beach, CA 92663, US</u>	<u>949-764-5600</u>			
CBC WITH DIFFERENTIAL (CBC AND DIFFERENTIAL - WAM AND NON-WAM) - Final result (02/11/2024 10:19 AM PST)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>WBC (White Blood Cells)</u>	<u>6.78</u>	<u>4.80 - 11.00 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Red Blood Cells</u>	<u>4.77</u>	<u>4.20 - 6.20 M/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Hemoglobin</u>	<u>14.4</u>	<u>14.0 - 18.0 g/dL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Hematocrit</u>	<u>43.7</u>	<u>42.0 - 54.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>MCV</u>	<u>91.6</u>	<u>80.0 - 100.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>MCH</u>	<u>30.2</u>	<u>27.0 - 31.0 pg</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>MCHC</u>	<u>33.0</u>	<u>32.0 - 37.0 g/dL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Platelet Count</u>	<u>238</u>	<u>150 - 400 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>RDW-SD</u>	<u>40.1</u>	<u>36.0 - 55.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>RDWCV</u>	<u>11.9</u>	<u>11.5 - 14.5 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>MPV</u>	<u>9.3</u>	<u>9.0 - 13.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% nRBC</u>	<u>0</u>	<u><1 per 100 WBCs</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Neutrophils</u>	<u>65.0</u>	<u>36.0 - 75.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>% Lymphocytes</u>	<u>26.4</u>	<u>13.0 - 50.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Monocytes</u>	<u>6.6</u>	<u>2.0 - 10.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Eosinophils</u>	<u>1.3</u>	<u>0.0 - 6.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Basophils</u>	<u>0.4</u>	<u>0.0 - 2.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Immature Granulocytes</u>	<u>0.3</u>	<u>0.0 - 0.8 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Neutrophils</u>	<u>4.40</u>	<u>1.00 - 7.00 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Lymphocytes</u>	<u>1.79</u>	<u>1.50 - 4.00 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Monocytes</u>	<u>0.45</u>	<u>0.20 - 0.90 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Eosinophils	0.09	0.00 - 0.70 K/uL		02/11/2024 10:32 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Absolute Basophils	0.03	0.00 - 0.20 K/uL		02/11/2024 10:32 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Absolute Immature Granulocytes	0.02	0.00 - 0.10 K/uL		02/11/2024 10:32 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume		Collection Time	Received Time
Blood	Venous blood specimen / Unknown		Venipuncture / Unknown		02/11/2024 10:19 AM PST	02/11/2024 10:24 AM PST
Narrative						
—						
Authorizing Provider	Result Type		Result Status			
Dr. H Marino DO	LAB BLOOD ORDERABLES		Final Result			
Performing Organization	Address		City/State/ZIP Code		Phone Number	
HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	One Hoag Drive		Newport Beach, CA 92663, US		949-764-5600	
COMPREHENSIVE METABOLIC PANEL - Final result (02/11/2024 10:19 AM PST)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium (Na)	139	135 - 145 mmol/L		02/11/2024 10:48 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Potassium (K)	4.0	3.5 - 5.2 mmol/L		02/11/2024 10:48 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Chloride (Cl)</u>	<u>106</u>	<u>100 - 110</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Carbon Dioxide (CO2)</u>	<u>27</u>	<u>24 - 32</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Anion Gap</u>	<u>6</u>	<u>6 - 14</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Glucose</u>	<u>97</u>	<u>65 - 99</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>BUN (Blood Urea Nitrogen)</u>	<u>17</u>	<u>9 - 20</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Creatinine</u>	<u>0.70</u>	<u>0.40 - 1.50</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>eGFR (Estimated Glomerular Filtration Rate)</u>	<u>>60</u>	<u>>=60</u> <u>mL/min/1.73m2</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Comment:

GFR value was calculated using a new GFR equation effective since 1/18/22. This is a recommendation of the American Society of Nephrology and National Kidney Foundation.

Clinical practice guidelines suggest the use of serum cystatin C as a confirmatory test for eGFR 45-59 mL/min/1.73m2 in adults who do not have markers of kidney disease; eGFR may be less accurate in this range.

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Calcium (Ca)</u>	<u>8.7</u>	<u>8.4 - 10.2</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Albumin</u>	<u>4.0</u>	<u>3.5 - 5.0</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Bilirubin, Total</u>	<u>0.5</u>	<u>0.2 - 1.3</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Total Protein</u>	<u>6.6</u>	<u>6.3 - 8.2</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>AST</u>	<u>32</u>	<u>17 - 59</u> <u>U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>ALT (Alanine Aminotransferase)(SGPT)</u>	<u>34</u>	<u><50 U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Alkaline Phosphatase</u>	<u>64</u>	<u>38 - 126</u> <u>U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Globulin</u>	<u>2.6</u>	<u>2.0 - 4.0</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Albumin/Globulin Ratio</u>	<u>1.5</u>	<u>1.2 - 2.0</u>		<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>BUN/Creatinine Ratio</u>	<u>24.3</u>			<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Specimen (Source)</u>	<u>Anatomical Location / Laterality</u>	<u>Collection Method / Volume</u>		<u>Collection Time</u>	<u>Received Time</u>	
<u>Blood</u>	<u>Venous blood specimen / Unknown</u>	<u>Venipuncture / Unknown</u>		<u>02/11/2024 10:19 AM PST</u>	<u>02/11/2024 10:24 AM PST</u>	
<u>Narrative</u>						
—						
<u>Authorizing Provider</u>	<u>Result Type</u>	<u>Result Status</u>				
<u>Dr. H Marino DO</u>	<u>LAB BLOOD ORDERABLES</u>	<u>Final Result</u>				
<u>Performing Organization</u>	<u>Address</u>	<u>City/State/ZIP Code</u>	<u>Phone Number</u>			
<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	<u>One Hoag Drive</u>	<u>Newport Beach, CA 92663, US</u>	<u>949-764-5600</u>			
<u>12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (02/11/2024 10:07 AM PST)</u>						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>VENTRICULAR RATE EKG</u>	<u>72</u>	<u>bpm</u>			<u>HOAG CV IECG RESULTING</u>	
<u>ATRIAL RATE</u>	<u>71</u>	<u>bpm</u>			<u>HOAG CV IECG RESULTING</u>	
<u>P-R INTERVAL</u>	<u>167</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>P WAVE AXIS</u>	<u>50</u>	<u>deg</u>			<u>HOAG CV IECG RESULTING</u>	
<u>QRS DURATION</u>	<u>98</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>Q-T INTERVAL</u>	<u>446</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>488</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>QRS AXIS</u>	<u>63</u>	<u>deg</u>			<u>HOAG CV IECG RESULTING</u>	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
T AXIS	58	deg			HOAG CV IECG RESULTING	
INTERPRETATION TEXT	- BORDERLINE ECG - Sinus rhythm Borderline prolonged QT interval when compared with ECG of 28-Mar-2023 17:35:28, significant rate decrease				HOAG CV IECG RESULTING	
INTERPRETATION TEXT	Electronically signed by: Kuo, Allen 11- Feb-2024 19:45:41				HOAG CV IECG RESULTING	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
				02/11/2024 10:07 AM PST		
Narrative						
This result has an attachment that is not available.						
-						
Authorizing Provider	Result Type	Result Status				
Dr. H Marino DO	ECG ORDERABLES	Final Result				
Performing Organization	Address	City/State/ZIP Code	Phone Number			
HOAG CV IECG RESULTING			-			
X-Ray (XR) Chest Anterior/Posterior Portable (XR CHEST AP PORTABLE) - Final result (02/11/2024 10:05 AM PST)						
Anatomical Region	Laterality	Modality				
Chest		Digital Radiography				
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
				02/11/2024 10:13 AM PST		
Impressions						
02/11/2024 10:14 AM PST						
1. No evidence of acute cardiopulmonary disease.						
Electronically signed by Humberto wong 2/11/2024 10:14 AM						
Narrative						
02/11/2024 10:14 AM PST						

Narrative

CHEST 1 VIEW 2/11/2024 9:59 AM

CLINICAL HISTORY: Pain

COMPARISON: 6/9/2023

FINDINGS:

Lungs are clear. Cardiomediastinal silhouette is normal. No pleural effusions or pneumothorax. No acute bony abnormalities

Procedure Note

Dr. H Wong, MD - 02/11/2024

Formatting of this note might be different from the original.

CHEST 1 VIEW 2/11/2024 9:59 AM

CLINICAL HISTORY: Pain

COMPARISON: 6/9/2023

FINDINGS:

Lungs are clear. Cardiomediastinal silhouette is normal. No pleural effusions or pneumothorax. No acute bony abnormalities

IMPRESSION:

1. No evidence of acute cardiopulmonary disease.

Electronically signed by Humberto Wong 2/11/2024 10:14 AM

Authorizing Provider	Result Type	Result Status
Dr. H Marino DO	IMG XR PROCEDURES	Final Result

XR CHEST 2 VW - Final result (06/09/2023 7:18 PM PDT)

Anatomical Region	Laterality	Modality
Chest		Radiographic Imaging

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

06/09/2023 7:24 PM PDT

1. No acute chest abnormality.

Electronically signed by Ashley Lotfipour 6/9/2023 7:24 PM

Narrative

06/09/2023 7:24 PM PDT

CHEST, PA AND LATERAL

CLINICAL HISTORY: RIB PAIN
MEDICATION REFILL.Pt injured left rib area after E bike accident. States pain is worse after moving furniture. Need refill for pain meds.

COMPARISON: 3/28/2023

FINDINGS:
The cardiomediastinum, pulmonary parenchyma, pleura, and osseous thorax demonstrate no acute abnormality. Old fracture deformity of the left ribs again seen.

Procedure Note

Dr. A Lotfipour, MD - 07/06/2023
Formatting of this note might be different from the original.

CHEST, PA AND LATERAL

CLINICAL HISTORY: RIB PAIN
MEDICATION REFILL.Pt injured left rib area after E bike accident. States pain is worse after moving furniture. Need refill for pain meds.

COMPARISON: 3/28/2023

FINDINGS:
The cardiomediastinum, pulmonary parenchyma, pleura, and osseous thorax demonstrate no acute abnormality. Old fracture deformity of the left ribs again seen.

IMPRESSION:
1. No acute chest abnormality.

Electronically signed by Ashley Lotfipour 6/9/2023 7:24 PM

Authorizing Provider	Result Type	Result Status
K Claunch PA	IMG XR PROCEDURES	Final Result

12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (03/28/2023 11:59 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG	117	bpm		03/29/2023 3:33 PM PDT	CA HH IECG	
ATRIAL RATE	116	bpm		03/29/2023 3:33 PM PDT	CA HH IECG	
P-R INTERVAL	149	ms		03/29/2023 3:33 PM PDT	CA HH IECG	
P WAVE AXIS	71	deg		03/29/2023 3:33 PM PDT	CA HH IECG	
QRS DURATION	74	ms		03/29/2023 3:33 PM PDT	CA HH IECG	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Q-T INTERVAL</u>	<u>340</u>	<u>ms</u>		<u>03/29/2023</u> <u>3:33 PM</u> <u>PDT</u>	<u>CA HH</u> <u>IECG</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>473</u>	<u>ms</u>		<u>03/29/2023</u> <u>3:33 PM</u> <u>PDT</u>	<u>CA HH</u> <u>IECG</u>	
<u>QRS AXIS</u>	<u>75</u>	<u>deg</u>		<u>03/29/2023</u> <u>3:33 PM</u> <u>PDT</u>	<u>CA HH</u> <u>IECG</u>	
<u>T AXIS</u>		<u>deg</u>		<u>03/29/2023</u> <u>3:33 PM</u> <u>PDT</u>	<u>CA HH</u> <u>IECG</u>	
<u>INTERPRETATION TEXT</u>	<u>- BORDERLINE</u> <u>ECG -</u> <u>Sinus</u> <u>tachycardia</u> <u>Borderline</u> <u>repolarization</u> <u>abnormality</u> <u>No previous</u> <u>ECG available</u> <u>for comparison</u> <u>Electronically</u> <u>signed by:</u> <u>Parto, Parham</u> <u>29-Mar-2023</u> <u>15:33:01</u>			<u>03/29/2023</u> <u>3:33 PM</u> <u>PDT</u>	<u>CA HH</u> <u>IECG</u>	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			<u>03/28/2023 5:35 PM PDT</u>	

Narrative

This result has an attachment that is not available.

—

Authorizing Provider	Result Type	Result Status	
<u>B Felter NP</u>	<u>ECG ORDERABLES</u>	<u>Final Result</u>	
Performing Organization	Address	City/State/ZIP Code	Phone Number
<u>HMHP CONVERSION</u>			-
<u>IMAGING</u>			-
<u>CA HH IECG</u>			-

XR CHEST 2 VW - Final result (03/28/2023 6:30 PM PDT)

Anatomical Region	Laterality	Modality
<u>Chest</u>		<u>Radiographic Imaging</u>

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/28/2023 6:50 PM PDT

- Trace left effusion.
- known left-sided rib fractures which are better seen on the recent CT

Electronically signed by Scott williams 3/28/2023 6:50 PM

03/28/2023 6:50 PM PDT

CHEST, PA AND LATERAL

CLINICAL HISTORY: Lt rib fx. worsening pain.

COMPARISON: CT 3/24/2023

FINDINGS:

Left-sided rib fractures are better seen on the recent CT. There is some trace fluid at the left base. No pneumothorax. Heart size is normal.

Procedure Note

Dr. S Williams, MD - 07/01/2023

Formatting of this note might be different from the original.

CHEST, PA AND LATERAL

CLINICAL HISTORY: Lt rib fx. worsening pain.

COMPARISON: CT 3/24/2023

FINDINGS:

Left-sided rib fractures are better seen on the recent CT. There is some trace fluid at the left base. No pneumothorax. Heart size is normal.

IMPRESSION:

- 1. Trace left effusion.
- 2. Known left-sided rib fractures which are better seen on the recent CT

Electronically signed by Scott Williams 3/28/2023 6:50 PM

Authorizing Provider	Result Type	Result Status
B Felter NP	IMG XR PROCEDURES	Final Result

Computed Tomography (CT) Chest Abdomen Pelvis Without Contrast (CT CHEST ABDOMEN PELVIS WO CONTRAST) - Final result (03/24/2023 1:51 AM PDT)

Anatomical Region	Laterality	Modality		
<u>Body, Chest</u>		<u>Computed Tomography</u>		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time

Impressions

03/24/2023 8:06 AM PDT

Impressions

1. Non to mildly displaced fractures of the left 3rd through 6th ribs without evidence for pneumothorax.
2. 1.2 cm peribronchial density in the left upper lobe probably infectious/inflammatory given associated bronchiectasis. 1.8 cm peri-fissural density in the medial right lower lobe. Recommend follow-up CT chest.

Electronically signed by Peter Giuliano 3/24/2023 8:06 AM

Narrative

03/24/2023 8:06 AM PDT

CT CHEST, ABDOMEN, AND PELVIS WITHOUT CONTRAST

CLINICAL HISTORY: Fall. Left-sided chest and abdominal pain

COMPARISON: None.

TECHNIQUE: Using a multidetector helical CT scanner, thin slice acquisition was performed through the chest, abdomen, and pelvis without intravenous contrast. Multiplanar reformations were obtained. CT scan done according to ALARA.

DOSE INFORMATION:

CTDIvol (mGy)

Series 2 - Spiral - 12.84 (mGy)

Total DLP (mGy-cm): 885.50 mGy.cm.

(Note: The above reported CTDIvol and DLP values are CT scanner radiation output related dose indices, and, as such, they do NOT represent actual patient dose estimates. A medical physicist should be consulted for specific questions regarding the radiation dose for this exam).

Note that evaluation of the solid organs, soft tissues, and vascular structures is limited without intravenous contrast.

CHEST FINDINGS:

Airways: Unremarkable.

Lungs: 1.8 cm x 0.5 cm infiltrate in the right lower lobe posterior to the heart on image 107 series 3. Small nodular infiltrate measuring just over centimeter in the left upper lobe on image 88 series 3 with adjacent bronchiectasis

Pleura: No effusion or thickening.

Heart and Pericardium: Heart size is normal. No pericardial effusion.

Coronary Artery Calcification: Absent.

Vessels: Unremarkable.

Mediastinum and Hila: Unremarkable.

Chest wall and Lower Neck: Unremarkable.

ABDOMEN AND PELVIS FINDINGS:

Liver: Unremarkable.

Gallbladder and Biliary Tree: Unremarkable.

Pancreas: Unremarkable.

Spleen: Unremarkable.

Adrenals: Unremarkable.

Kidneys and Ureters: Unremarkable.

Bladder: Unremarkable.

Reproductive Organs: Unremarkable.

Bowel: Unremarkable.

Lymph Nodes: No obvious lymphadenopathy.

Peritoneum: No free air, free fluid, or fluid collections.

Vessels: Unremarkable.

Abdominal wall: Unremarkable.

BONE FINDINGS: Non to minimally displaced fractures of the lateral/anterolateral left 3rd through 6th ribs

—

Dr. P Giuliano, MD - 07/01/2023

Formatting of this note might be different from the original.

CT CHEST, ABDOMEN, AND PELVIS WITHOUT CONTRAST

CLINICAL HISTORY: Fall. Left-sided chest and abdominal pain

COMPARISON: None.

TECHNIQUE: Using a multidetector helical CT scanner, thin slice acquisition was performed through the chest, abdomen, and pelvis without intravenous contrast. Multiplanar reformations were obtained. CT scan done according to ALARA.

DOSE INFORMATION:

CTDIvol (mGy)

Series 2 - Spiral - 12.84 (mGy)

Total DLP (mGy-cm): 885.50 mGy.cm.

(Note: The above reported CTDIvol and DLP values are CT scanner radiation output related dose indices, and, as such, they do NOT represent actual patient dose estimates. A medical physicist should be consulted for specific questions regarding the radiation dose for this exam).

Note that evaluation of the solid organs, soft tissues, and vascular structures is limited without intravenous contrast.

CHEST FINDINGS:

Airways: Unremarkable.

Lungs: 1.8 cm x 0.5 cm infiltrate in the right lower lobe posterior to the heart on image 107 series 3. Small nodular infiltrate measuring just over centimeter in the left upper lobe on image 88 series 3 with adjacent bronchiectasis

Pleura: No effusion or thickening.

Heart and Pericardium: Heart size is normal. No pericardial effusion.

Coronary Artery Calcification: Absent.

Vessels: Unremarkable.

Mediastinum and Hila: Unremarkable.

Chest Wall and Lower Neck: Unremarkable.

ABDOMEN AND PELVIS FINDINGS:

Liver: Unremarkable.

Gallbladder and Biliary Tree: Unremarkable.

Pancreas: Unremarkable.

Spleen: Unremarkable.

Adrenals: Unremarkable.

Kidneys and Ureters: Unremarkable.

Bladder: Unremarkable.

Reproductive Organs: Unremarkable.

Bowel: Unremarkable.

Lymph Nodes: No obvious lymphadenopathy.

Peritoneum: No free air, free fluid, or fluid collections.

Procedure Note

Vessels: Unremarkable.

Abdominal Wall: Unremarkable.

BONE FINDINGS: Non to minimally displaced fractures of the lateral/anterolateral left 3rd through 6th ribs

IMPRESSION:

- 1. Non to mildly displaced fractures of the left 3rd through 6th ribs without evidence for pneumothorax.
- 2. 1.2 cm peribronchial density in the left upper lobe probably infectious/inflammatory given associated bronchiectasis. 1.8 cm peri-fissural density in the medial right lower lobe. Recommend follow-up CT chest.

Electronically signed by Peter Giuliano 3/24/2023 8:06 AM

Authorizing Provider	Result Type	Result Status
B Felter NP	IMG CT PROCEDURES	Final Result

XR SHOULDER LEFT 2 PLUS VW - Final result (03/24/2023 1:27 AM PDT)

Anatomical Region	Laterality	Modality
Upper Extremities, Shoulder	Left	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:44 AM PDT

1. Multiple minimally displaced left-sided rib fractures. No significant pneumothorax No acute osseous abnormality of the left shoulder.

Electronically signed by Peter Giuliano 3/24/2023 7:44 AM

Narrative

03/24/2023 7:44 AM PDT

LEFT SHOULDER, 4 VIEWS

CLINICAL HISTORY: Fall. Pain. COMPARISON: None.

FINDINGS:

Several nondisplaced fractures of the lateral left rib cage noted approximately at the level of the 5th through 7th ribs No other acute fracture or dislocation. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

Procedure Note

Dr. P Giuliano, MD - 07/01/2023

Formatting of this note might be different from the original.

LEFT SHOULDER, 4 VIEWS

CLINICAL HISTORY: Fall. Pain. COMPARISON: None.

FINDINGS:

Several nondisplaced fractures of the lateral left rib cage noted approximately at the level of the 5th through 7th ribs No other acute fracture or dislocation. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

IMPRESSION:

1. Multiple minimally displaced left-sided rib fractures. No significant pneumothorax No acute osseous abnormality of the left shoulder.

Electronically signed by Peter Giuliano 3/24/2023 7:44 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

X-Ray (XR) Ankle Right 3 or more views (XR ANKLE 3+ VW RIGHT) - Final result (03/24/2023 1:27 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Right	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:44 AM PDT

1. No acute osseous abnormality of the right ankle.

Electronically signed by Shaya Ghazinoor 3/24/2023 7:44 AM

Narrative

03/24/2023 7:44 AM PDT

RIGHT ANKLE, 3 VIEWS

CLINICAL HISTORY: Ankle pain status post fall.

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The ankle mortise is symmetric and the other joint spaces are preserved. No significant soft tissue swelling.

Formatting of this note might be different from the original.

RIGHT ANKLE, 3 VIEWS

CLINICAL HISTORY: Ankle pain status post fall.

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The ankle mortise is symmetric and the other joint spaces are preserved. No significant soft tissue swelling.

IMPRESSION:

1. No acute osseous abnormality of the right ankle.

Electronically signed by Shaya Ghazinoor 3/24/2023 7:44 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

X-Ray (XR) Tibia Fibula Right 2 views (XR TIBIA FIBULA 2 VIEWS RIGHT) - Final result (03/24/2023 1:26 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Lower Leg	Right	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	-------------------------------------	-------------------------------	-----------------	---------------

Impressions

03/24/2023 7:42 AM PDT

1. No acute osseous abnormality of the right tibia and fibula.

Electronically signed by Peter Giuliano 3/24/2023 7:42 AM

Narrative

03/24/2023 7:42 AM PDT

RIGHT TIBIA AND FIBULA, 2 VIEWS

CLINICAL HISTORY: Pain midshaft; fall off bike with pain

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The joint spaces are preserved. No significant soft tissue swelling. No radiopaque foreign body

Formatting of this note might be different from the original.

RIGHT TIBIA AND FIBULA, 2 VIEWS

CLINICAL HISTORY: Pain midshaft; fall off bike with pain

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The joint spaces are preserved. No significant soft tissue swelling. No radiopaque foreign body

IMPRESSION:

1. No acute osseous abnormality of the right tibia and fibula.

Electronically signed by Peter Giuliano 3/24/2023 7:42 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

Care Teams

Team Member	Relationship	Specialty	Start Date	End Date
PCp Not Assigned NPI: 1100007594 One Hoag Drive Newport Beach, CA 92663 949-764-4624 (Work)	PCP - General		2/11/24	

Document Information

Primary Care Provider	Other Service Providers	Document Coverage Dates
PCp Not Assigned (Feb. 11, 2024 - Present) NPI: 1100007594 949-764-4624 (Work) One Hoag Drive Newport Beach, CA 92663 HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		May 04, 1983 - Aug. 29, 2026
Custodian Organization		
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		
Legal Authenticator		
Director H		



If you take your Lucy record on a thumb drive to a different doctor, he or she might be able to use his computer to read the file electronically. Your downloaded, machine-readable Personal Health Summary document is in a format called "CDA." If your doctor has a computer that understands CDA, your information is a folder on your thumb drive called MachineReadable_XDMFormat. You might need to enter a password before your doctor can use this file.

Anthony Dimarcello "Anthony"

Continuity of Care Document, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
<i>Former (Oct. 30, 2023 - Feb. 10, 2024):</i> 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
<i>(Feb. 24, 2026 -):</i> 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	<i>Former / Aliases:</i> Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
<i>Former (Aug. 29, 2024 - Feb. 23, 2026):</i> 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
<i>Former (Feb. 11, 2024 - Aug. 28, 2024):</i> Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Encounters - as of 08/29/2026

Date	Type	Department	Care Team	Description
09/14/2024 3:39 PM PDT - 09/14/2024 5:06 PM PDT	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	C Caputo, PA-C Dr. C Goldsworthy, MD	Discharge Disposition: Home or Self Care
08/29/2024 9:55 PM PDT - 08/29/2024 10:56 PM PDT	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	M Willenborg, PA-C Dr. D Ritter, MD	Discharge Disposition: AMA
08/29/2024 2:54 AM PDT - 08/29/2024 6:28 AM PDT	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	Dr. E Kang, MD	Discharge Disposition: Home or Self Care
02/11/2024 9:46 AM PST - 02/11/2024 12:58 PM PST	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	Dr. H Marino, DO	Discharge Disposition: Home or Self Care
06/09/2023 6:52 PM PDT - 06/09/2023 8:32 PM PDT	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	Dr. E Alcouloumre, MD	Discharge Disposition: Home or Self Care
03/28/2023 5:25 PM PDT - 03/28/2023 11:19 PM PDT	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	Dr. A Cunningham, MD	Discharge Disposition: Home or Self Care
03/24/2023 12:20 AM PDT - 03/24/2023 3:20 AM PDT	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	Dr. D Ritter, MD	Discharge Disposition: LEFT AGAINST MEDICAL ADVICE OR DISCONTINUED CARE
08/20/2014 1:37 PM PDT - 08/20/2014 3:00 PM PDT	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	PROVIDER, HOAG IP CONVERSION	PROVIDER, HOAG IP CONVERSION Discharge Disposition: Home or Self Care

Allergies - as of 08/29/2026

No known active allergies

Medications - as of 08/29/2026

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date	Status
<u>HYDROcodone-acetaminophen (NORCO) 10-325 mg tablet</u>	<u>Take 0.5 tablets by mouth every 6 (six) hours if needed for moderate pain (4-6) for up to 12 doses</u>	6 tablet		09/14/2024		Active

Active Problems - as of 08/29/2026

Not on file

Social History - as of 08/29/2026

Smoking Status as of 09/14/2024

Tobacco Use	Types	Packs/Day	Years Used	Date
-------------	-------	-----------	------------	------

Smoking Tobacco: Some Days

Smoking Status as of 02/11/2024

Tobacco Use	Types	Packs/Day	Years Used	Date
-------------	-------	-----------	------------	------

Smoking Tobacco: Some Days

Housing Stability	Answer	Date Recorded
-------------------	--------	---------------

Housing Stability	Not on file	08/05/2023
-------------------	-------------	------------

Housing Stability	Not on file	08/05/2023
-------------------	-------------	------------

Interpersonal Safety	Answer	Date Recorded
----------------------	--------	---------------

Interpersonal Safety	Not on file	12/01/2024
----------------------	-------------	------------

Interpersonal Safety	Not on file	12/01/2024
----------------------	-------------	------------

Interpersonal Safety	Not on file	12/01/2024
----------------------	-------------	------------

Interpersonal Safety	Not on file	12/01/2024
----------------------	-------------	------------

Sex and Gender Information	Value	Date Recorded
----------------------------	-------	---------------

<u>Sex Assigned at Birth</u>	<u>Male</u>	<u>01/22/2025 10:41 AM PST</u>
------------------------------	-------------	--------------------------------

<u>Legal Sex</u>	<u>Male</u>	<u>05/31/2023 8:08 PM PDT</u>
------------------	-------------	-------------------------------

<u>Gender Identity</u>	<u>Male</u>	<u>01/22/2025 10:41 AM PST</u>
------------------------	-------------	--------------------------------

<u>Sexual Orientation</u>	<u>Straight</u>	<u>01/22/2025 10:41 AM PST</u>
---------------------------	-----------------	--------------------------------

Last Filed Vital Signs - as of 08/29/2026

Not on file

Plan of Treatment - as of 08/29/2026

Not on file

Results - as of 08/29/2026

X-Ray (XR) Left Shoulder 2 or More Views (XR SHOULDER LEFT 2+ VW) - Final result (09/14/2024 4:06 PM PDT)

Anatomical Region	Laterality	Modality
Shoulder	Left	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			09/14/2024 4:08 PM PDT	

Impressions

09/14/2024 4:10 PM PDT

1. Redemonstrated comminuted fracture of the left humeral neck.

Electronically signed by January Lopez 9/14/2024 4:10 PM

Narrative

09/14/2024 4:10 PM PDT

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT SHOULDER, 4 views

CLINICAL HISTORY: Left shoulder pain after trauma.

COMPARISON: August 29, 2024.

FINDINGS:

There is redemonstration of a comminuted fracture through the left humeral neck with slight lateral displacement of the shaft relative to the head. There is evidence of hemarthrosis with widening of the subacromial space. The acromioclavicular joint is intact.

—

Procedure Note

Dr. J Lopez, MD - 09/14/2024

Formatting of this note might be different from the original.

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT SHOULDER, 4 views

CLINICAL HISTORY: Left shoulder pain after trauma.

COMPARISON: August 29, 2024.

FINDINGS:

There is redemonstration of a comminuted fracture through the left humeral neck with slight lateral displacement of the shaft relative to the head. There is evidence of hemarthrosis with widening of the subacromial space. The acromioclavicular joint is intact.

—

IMPRESSION:

1. Redemonstrated comminuted fracture of the left humeral neck.

Electronically signed by January Lopez 9/14/2024 4:10 PM

Authorizing Provider	Result Type	Result Status
C Caputo PA-C	IMG XR PROCEDURES	Final Result

X-Ray (XR) Chest 1 View (XR CHEST 1 VW) - Final result (08/29/2024 9:58 PM PDT)

Anatomical Region	Laterality	Modality
Chest		Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time		
			08/29/2024 10:12 PM PDT			
Impressions						
08/29/2024 10:12 PM PDT						
1. No evidence of acute cardiopulmonary disease.						
Electronically signed by John Lieu 8/29/2024 10:12 PM						
Narrative						
08/29/2024 10:12 PM PDT						
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO						
CHEST, 1 VIEW						
CLINICAL HISTORY: TRAUMA.						
COMPARISON: Chest x-ray 2/11/2024						
FINDINGS:						
The lungs are clear. The heart size is within normal limits.						
No acute osseous process.						
Procedure Note						
Dr. J Lieu, MD - 08/29/2024						
Formatting of this note might be different from the original.						
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO						
CHEST, 1 VIEW						
CLINICAL HISTORY: TRAUMA.						
COMPARISON: Chest x-ray 2/11/2024						
FINDINGS:						
The lungs are clear. The heart size is within normal limits.						
No acute osseous process.						
IMPRESSION:						
1. No evidence of acute cardiopulmonary disease.						
Electronically signed by John Lieu 8/29/2024 10:12 PM						
Authorizing Provider	Result Type	Result Status				
Dr. J Houpt MD	IMG XR PROCEDURES	Final Result				
12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (08/29/2024 8:14 PM PDT)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG	89	bpm			HOAG CV IECG RESULTING	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>ATRIAL RATE</u>	<u>87</u>	<u>bpm</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>P-R INTERVAL</u>	<u>134</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>P WAVE AXIS</u>	<u>76</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>QRS DURATION</u>	<u>97</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Q-T INTERVAL</u>	<u>373</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>455</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>QRS AXIS</u>	<u>56</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>T AXIS</u>	<u>45</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>INTERPRETATION TEXT</u>	<u>- NORMAL</u> <u>ECG -</u> <u>Sinus</u> <u>rhythm</u> <u>when</u> <u>compared</u> <u>with ECG of</u> <u>11-Feb-2024</u> <u>10:07:33,</u> <u>Nonspecific</u> <u>significant</u> <u>change</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>INTERPRETATION TEXT</u>	<u>Electronically</u> <u>signed by:</u> <u>Gandhi,</u> <u>Radhakrishan</u> <u>29-Aug-2024</u> <u>22:10:05</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Specimen (Source)</u>	<u>Anatomical Location /</u> <u>Laterality</u>		<u>Collection Method /</u> <u>Volume</u>		<u>Collection Time</u>	<u>Received Time</u>
					<u>08/29/2024 8:14 PM PDT</u>	
<u>Narrative</u>						
<u>This result has an attachment that is not available.</u>						
<u>-</u>						
<u>Authorizing Provider</u>	<u>Result Type</u>		<u>Result Status</u>			
<u>Dr. J Houpt MD</u>	<u>ECG ORDERABLES</u>		<u>Final Result</u>			
<u>Performing Organization</u>	<u>Address</u>		<u>City/State/ZIP Code</u>	<u>Phone Number</u>		
<u>HOAG CV IECG</u> <u>RESULTING</u>				<u>-</u>		
<u>X-Ray (XR) Left Humerus 2 or More Views (XR HUMERUS LEFT 2+ VW) - Final result (08/29/2024 3:56 AM PDT)</u>						
<u>Anatomical Region</u>		<u>Laterality</u>		<u>Modality</u>		
<u>Upper Extremities</u>		<u>Left</u>		<u>Digital Radiography</u>		

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			08/29/2024 7:47 AM PDT	
Impressions				
08/29/2024 7:48 AM PDT				
1. Acute mildly displaced proximal left humerus fracture.				
Electronically signed by Gerard Nguyen 8/29/2024 7:48 AM				
Narrative				
08/29/2024 7:48 AM PDT				
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO				
LEFT HUMERUS, 2 VIEWS				
CLINICAL HISTORY: Left shoulder pain.				
COMPARISON: None.				
FINDINGS:				
Acute proximal humerus neck fracture with lateral displacement approximately one half shaft width. The joint spaces are preserved. No significant soft tissue swelling.				
Procedure Note				
Dr. G Nguyen, MD - 08/29/2024				
Formatting of this note might be different from the original.				
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO				
LEFT HUMERUS, 2 VIEWS				
CLINICAL HISTORY: Left shoulder pain.				
COMPARISON: None.				
FINDINGS:				
Acute proximal humerus neck fracture with lateral displacement approximately one half shaft width. The joint spaces are preserved. No significant soft tissue swelling.				
IMPRESSION:				
1. Acute mildly displaced proximal left humerus fracture.				
Electronically signed by Gerard Nguyen 8/29/2024 7:48 AM				
Authorizing Provider	Result Type	Result Status		
Dr. E Kang MD	IMG XR PROCEDURES	Final Result		
X-Ray (XR) Left Shoulder 2 or More Views (XR SHOULDER LEFT 2+ VW) - Final result (08/29/2024 3:55 AM PDT)				
Anatomical Region	Laterality	Modality		
Shoulder	Left	Digital Radiography		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			08/29/2024 7:48 AM PDT	

08/29/2024 7:49 AM PDT

1. Acute mildly displaced proximal left humerus fracture.

Electronically signed by Gerard Nguyen 8/29/2024 7:49 AM

Narrative

08/29/2024 7:49 AM PDT

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

XR SHOULDER LEFT 2+ VW

CLINICAL HISTORY: Left shoulder pain.

COMPARISON: None.

FINDINGS:

Acute comminuted proximal humerus neck fracture with lateral displacement approximately one half shaft width. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

Procedure Note

Dr. G Nguyen, MD - 08/29/2024

Formatting of this note might be different from the original.

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

XR SHOULDER LEFT 2+ VW

CLINICAL HISTORY: Left shoulder pain.

COMPARISON: None.

FINDINGS:

Acute comminuted proximal humerus neck fracture with lateral displacement approximately one half shaft width. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

IMPRESSION:

1. Acute mildly displaced proximal left humerus fracture.

Electronically signed by Gerard Nguyen 8/29/2024 7:49 AM

Authorizing Provider	Result Type	Result Status
Dr. E Kang MD	IMG XR PROCEDURES	Final Result

TROPONIN I - Final result (02/11/2024 10:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I	<0.01	0.00 - 0.03 ng/mL		02/11/2024 10:58 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time		
<u>Blood</u>	<u>Venous blood specimen / Unknown</u>	<u>Venipuncture / Unknown</u>	<u>02/11/2024 10:19 AM PST</u>	<u>02/11/2024 10:24 AM PST</u>		
Narrative						
—						
Authorizing Provider	Result Type	Result Status				
<u>Dr. H Marino DO</u>	<u>LAB BLOOD ORDERABLES</u>	<u>Final Result</u>				
Performing Organization	Address	City/State/ZIP Code	Phone Number			
<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	<u>One Hoag Drive</u>	<u>Newport Beach, CA 92663, US</u>	<u>949-764-5600</u>			
CBC WITH DIFFERENTIAL (CBC AND DIFFERENTIAL - WAM AND NON-WAM) - Final result (02/11/2024 10:19 AM PST)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>WBC (White Blood Cells)</u>	<u>6.78</u>	<u>4.80 - 11.00 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Red Blood Cells</u>	<u>4.77</u>	<u>4.20 - 6.20 M/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Hemoglobin</u>	<u>14.4</u>	<u>14.0 - 18.0 g/dL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Hematocrit</u>	<u>43.7</u>	<u>42.0 - 54.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>MCV</u>	<u>91.6</u>	<u>80.0 - 100.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>MCH</u>	<u>30.2</u>	<u>27.0 - 31.0 pg</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>MCHC</u>	<u>33.0</u>	<u>32.0 - 37.0 g/dL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Platelet Count</u>	<u>238</u>	<u>150 - 400 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>RDW-SD</u>	<u>40.1</u>	<u>36.0 - 55.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>RDWCV</u>	<u>11.9</u>	<u>11.5 - 14.5 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>MPV</u>	<u>9.3</u>	<u>9.0 - 13.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% nRBC</u>	<u>0</u>	<u><1 per 100 WBCs</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Neutrophils</u>	<u>65.0</u>	<u>36.0 - 75.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>% Lymphocytes</u>	<u>26.4</u>	<u>13.0 - 50.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Monocytes</u>	<u>6.6</u>	<u>2.0 - 10.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Eosinophils</u>	<u>1.3</u>	<u>0.0 - 6.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Basophils</u>	<u>0.4</u>	<u>0.0 - 2.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Immature Granulocytes</u>	<u>0.3</u>	<u>0.0 - 0.8 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Neutrophils</u>	<u>4.40</u>	<u>1.00 - 7.00 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Lymphocytes</u>	<u>1.79</u>	<u>1.50 - 4.00 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Monocytes</u>	<u>0.45</u>	<u>0.20 - 0.90 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Absolute Eosinophils	0.09	0.00 - 0.70 K/uL		02/11/2024 10:32 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Absolute Basophils	0.03	0.00 - 0.20 K/uL		02/11/2024 10:32 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Absolute Immature Granulocytes	0.02	0.00 - 0.10 K/uL		02/11/2024 10:32 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume		Collection Time	Received Time
Blood	Venous blood specimen / Unknown		Venipuncture / Unknown		02/11/2024 10:19 AM PST	02/11/2024 10:24 AM PST
Narrative						
—						
Authorizing Provider	Result Type		Result Status			
Dr. H Marino DO	LAB BLOOD ORDERABLES		Final Result			
Performing Organization	Address		City/State/ZIP Code	Phone Number		
HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	One Hoag Drive		Newport Beach, CA 92663, US	949-764-5600		
COMPREHENSIVE METABOLIC PANEL - Final result (02/11/2024 10:19 AM PST)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium (Na)	139	135 - 145 mmol/L		02/11/2024 10:48 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
Potassium (K)	4.0	3.5 - 5.2 mmol/L		02/11/2024 10:48 AM PST	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Chloride (Cl)</u>	<u>106</u>	<u>100 - 110</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Carbon Dioxide (CO2)</u>	<u>27</u>	<u>24 - 32</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Anion Gap</u>	<u>6</u>	<u>6 - 14</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Glucose</u>	<u>97</u>	<u>65 - 99</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>BUN (Blood Urea Nitrogen)</u>	<u>17</u>	<u>9 - 20</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Creatinine</u>	<u>0.70</u>	<u>0.40 - 1.50</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>eGFR (Estimated Glomerular Filtration Rate)</u>	<u>>60</u>	<u>>=60</u> <u>mL/min/1.73m2</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Comment:

GFR value was calculated using a new GFR equation effective since 1/18/22. This is a recommendation of the American Society of Nephrology and National Kidney Foundation.

Clinical practice guidelines suggest the use of serum cystatin C as a confirmatory test for eGFR 45-59 mL/min/1.73m2 in adults who do not have markers of kidney disease; eGFR may be less accurate in this range.

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Calcium (Ca)</u>	<u>8.7</u>	<u>8.4 - 10.2</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Albumin</u>	<u>4.0</u>	<u>3.5 - 5.0</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Bilirubin, Total</u>	<u>0.5</u>	<u>0.2 - 1.3</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Total Protein</u>	<u>6.6</u>	<u>6.3 - 8.2</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>AST</u>	<u>32</u>	<u>17 - 59</u> <u>U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>ALT (Alanine Aminotransferase)(SGPT)</u>	<u>34</u>	<u><50 U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Alkaline Phosphatase</u>	<u>64</u>	<u>38 - 126</u> <u>U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Globulin</u>	<u>2.6</u>	<u>2.0 - 4.0</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Albumin/Globulin Ratio</u>	<u>1.5</u>	<u>1.2 - 2.0</u>		<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>BUN/Creatinine Ratio</u>	<u>24.3</u>			<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Specimen (Source)</u>	<u>Anatomical Location / Laterality</u>	<u>Collection Method / Volume</u>		<u>Collection Time</u>	<u>Received Time</u>	
<u>Blood</u>	<u>Venous blood specimen / Unknown</u>	<u>Venipuncture / Unknown</u>		<u>02/11/2024 10:19 AM PST</u>	<u>02/11/2024 10:24 AM PST</u>	
<u>Narrative</u>						
—						
<u>Authorizing Provider</u>	<u>Result Type</u>	<u>Result Status</u>				
<u>Dr. H Marino DO</u>	<u>LAB BLOOD ORDERABLES</u>	<u>Final Result</u>				
<u>Performing Organization</u>	<u>Address</u>	<u>City/State/ZIP Code</u>	<u>Phone Number</u>			
<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	<u>One Hoag Drive</u>	<u>Newport Beach, CA 92663, US</u>	<u>949-764-5600</u>			
<u>12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (02/11/2024 10:07 AM PST)</u>						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>VENTRICULAR RATE EKG</u>	<u>72</u>	<u>bpm</u>			<u>HOAG CV IECG RESULTING</u>	
<u>ATRIAL RATE</u>	<u>71</u>	<u>bpm</u>			<u>HOAG CV IECG RESULTING</u>	
<u>P-R INTERVAL</u>	<u>167</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>P WAVE AXIS</u>	<u>50</u>	<u>deg</u>			<u>HOAG CV IECG RESULTING</u>	
<u>QRS DURATION</u>	<u>98</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>Q-T INTERVAL</u>	<u>446</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>488</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	
<u>QRS AXIS</u>	<u>63</u>	<u>deg</u>			<u>HOAG CV IECG RESULTING</u>	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
T AXIS	58	deg			HOAG CV IECG RESULTING	
INTERPRETATION TEXT	- BORDERLINE ECG - Sinus rhythm Borderline prolonged QT interval when compared with ECG of 28-Mar-2023 17:35:28, significant rate decrease				HOAG CV IECG RESULTING	
INTERPRETATION TEXT	Electronically signed by: Kuo, Allen 11- Feb-2024 19:45:41				HOAG CV IECG RESULTING	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
				02/11/2024 10:07 AM PST		
Narrative						
This result has an attachment that is not available.						
-						
Authorizing Provider	Result Type	Result Status				
Dr. H Marino DO	ECG ORDERABLES	Final Result				
Performing Organization	Address	City/State/ZIP Code	Phone Number			
HOAG CV IECG RESULTING			-			
X-Ray (XR) Chest Anterior/Posterior Portable (XR CHEST AP PORTABLE) - Final result (02/11/2024 10:05 AM PST)						
Anatomical Region	Laterality	Modality				
Chest		Digital Radiography				
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
				02/11/2024 10:13 AM PST		
Impressions						
02/11/2024 10:14 AM PST						
1. No evidence of acute cardiopulmonary disease.						
Electronically signed by Humberto wong 2/11/2024 10:14 AM						
Narrative						
02/11/2024 10:14 AM PST						

Narrative

CHEST 1 VIEW 2/11/2024 9:59 AM

CLINICAL HISTORY: Pain

COMPARISON: 6/9/2023

FINDINGS:

Lungs are clear. Cardiomediastinal silhouette is normal. No pleural effusions or pneumothorax. No acute bony abnormalities

Procedure Note

Dr. H Wong, MD - 02/11/2024

Formatting of this note might be different from the original.

CHEST 1 VIEW 2/11/2024 9:59 AM

CLINICAL HISTORY: Pain

COMPARISON: 6/9/2023

FINDINGS:

Lungs are clear. Cardiomediastinal silhouette is normal. No pleural effusions or pneumothorax. No acute bony abnormalities

IMPRESSION:

1. No evidence of acute cardiopulmonary disease.

Electronically signed by Humberto Wong 2/11/2024 10:14 AM

Authorizing Provider	Result Type	Result Status
Dr. H Marino DO	IMG XR PROCEDURES	Final Result

XR CHEST 2 VW - Final result (06/09/2023 7:18 PM PDT)

Anatomical Region	Laterality	Modality
Chest		Radiographic Imaging

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	-------------------------------------	-------------------------------	-----------------	---------------

Impressions

06/09/2023 7:24 PM PDT

1. No acute chest abnormality.

Electronically signed by Ashley Lotfipour 6/9/2023 7:24 PM

Narrative

06/09/2023 7:24 PM PDT

CHEST, PA AND LATERAL

CLINICAL HISTORY: RIB PAIN
MEDICATION REFILL.Pt injured left rib area after E bike accident. States pain is worse after moving furniture. Need refill for pain meds.

COMPARISON: 3/28/2023

FINDINGS:
The cardiomediatinum, pulmonary parenchyma, pleura, and osseous thorax demonstrate no acute abnormality. Old fracture deformity of the left ribs again seen.

Procedure Note

Dr. A Lotfipour, MD - 07/06/2023
Formatting of this note might be different from the original.

CHEST, PA AND LATERAL

CLINICAL HISTORY: RIB PAIN
MEDICATION REFILL.Pt injured left rib area after E bike accident. States pain is worse after moving furniture. Need refill for pain meds.

COMPARISON: 3/28/2023

FINDINGS:
The cardiomediatinum, pulmonary parenchyma, pleura, and osseous thorax demonstrate no acute abnormality. Old fracture deformity of the left ribs again seen.

IMPRESSION:
1. No acute chest abnormality.

Electronically signed by Ashley Lotfipour 6/9/2023 7:24 PM

Authorizing Provider	Result Type	Result Status
K Claunch PA	IMG XR PROCEDURES	Final Result

12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (03/28/2023 11:59 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
VENTRICULAR RATE EKG	117	bpm		03/29/2023 3:33 PM PDT	CA HH IECG	
ATRIAL RATE	116	bpm		03/29/2023 3:33 PM PDT	CA HH IECG	
P-R INTERVAL	149	ms		03/29/2023 3:33 PM PDT	CA HH IECG	
P WAVE AXIS	71	deg		03/29/2023 3:33 PM PDT	CA HH IECG	
QRS DURATION	74	ms		03/29/2023 3:33 PM PDT	CA HH IECG	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Q-T INTERVAL</u>	<u>340</u>	<u>ms</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>473</u>	<u>ms</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>QRS AXIS</u>	<u>75</u>	<u>deg</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>T AXIS</u>		<u>deg</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>INTERPRETATION TEXT</u>	<u>- BORDERLINE ECG - Sinus tachycardia Borderline repolarization abnormality No previous ECG available for comparison Electronically signed by: Parto, Parham 29-Mar-2023 15:33:01</u>			<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			<u>03/28/2023 5:35 PM PDT</u>	

Narrative

This result has an attachment that is not available.

—

Authorizing Provider	Result Type	Result Status	
<u>B Felter NP</u>	<u>ECG ORDERABLES</u>	<u>Final Result</u>	
Performing Organization	Address	City/State/ZIP Code	Phone Number
<u>HMHP CONVERSION IMAGING</u>			-
<u>CA HH IECG</u>			-

XR CHEST 2 VW - Final result (03/28/2023 6:30 PM PDT)

Anatomical Region	Laterality	Modality
<u>Chest</u>		<u>Radiographic Imaging</u>

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/28/2023 6:50 PM PDT

1. Trace left effusion.
2. known left-sided rib fractures which are better seen on the recent CT

Electronically signed by Scott williams 3/28/2023 6:50 PM

03/28/2023 6:50 PM PDT

CHEST, PA AND LATERAL

CLINICAL HISTORY: Lt rib fx. worsening pain.

COMPARISON: CT 3/24/2023

FINDINGS:

Left-sided rib fractures are better seen on the recent CT. There is some trace fluid at the left base. No pneumothorax. Heart size is normal.

Procedure Note

Dr. S Williams, MD - 07/01/2023

Formatting of this note might be different from the original.

CHEST, PA AND LATERAL

CLINICAL HISTORY: Lt rib fx. worsening pain.

COMPARISON: CT 3/24/2023

FINDINGS:

Left-sided rib fractures are better seen on the recent CT. There is some trace fluid at the left base. No pneumothorax. Heart size is normal.

IMPRESSION:

- 1. Trace left effusion.
- 2. Known left-sided rib fractures which are better seen on the recent CT

Electronically signed by Scott Williams 3/28/2023 6:50 PM

Authorizing Provider	Result Type	Result Status
B Felter NP	IMG XR PROCEDURES	Final Result

Computed Tomography (CT) Chest Abdomen Pelvis Without Contrast (CT CHEST ABDOMEN PELVIS WO CONTRAST) - Final result (03/24/2023 1:51 AM PDT)

Anatomical Region	Laterality	Modality		
<u>Body, Chest</u>		<u>Computed Tomography</u>		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time

Impressions

03/24/2023 8:06 AM PDT

Impressions

1. Non to mildly displaced fractures of the left 3rd through 6th ribs without evidence for pneumothorax.
2. 1.2 cm peribronchial density in the left upper lobe probably infectious/inflammatory given associated bronchiectasis. 1.8 cm peri-fissural density in the medial right lower lobe. Recommend follow-up CT chest.

Electronically signed by Peter Giuliano 3/24/2023 8:06 AM

Narrative

03/24/2023 8:06 AM PDT

CT CHEST, ABDOMEN, AND PELVIS WITHOUT CONTRAST

CLINICAL HISTORY: Fall. Left-sided chest and abdominal pain

COMPARISON: None.

TECHNIQUE: Using a multidetector helical CT scanner, thin slice acquisition was performed through the chest, abdomen, and pelvis without intravenous contrast. Multiplanar reformations were obtained. CT scan done according to ALARA.

DOSE INFORMATION:

CTDIvol (mGy)

Series 2 - Spiral - 12.84 (mGy)

Total DLP (mGy-cm): 885.50 mGy.cm.

(Note: The above reported CTDIvol and DLP values are CT scanner radiation output related dose indices, and, as such, they do NOT represent actual patient dose estimates. A medical physicist should be consulted for specific questions regarding the radiation dose for this exam).

Note that evaluation of the solid organs, soft tissues, and vascular structures is limited without intravenous contrast.

CHEST FINDINGS:

Airways: Unremarkable.

Lungs: 1.8 cm x 0.5 cm infiltrate in the right lower lobe posterior to the heart on image 107 series 3. Small nodular infiltrate measuring just over centimeter in the left upper lobe on image 88 series 3 with adjacent bronchiectasis

Pleura: No effusion or thickening.

Heart and Pericardium: Heart size is normal. No pericardial effusion.

Coronary Artery Calcification: Absent.

Vessels: Unremarkable.

Mediastinum and Hila: Unremarkable.

Chest wall and Lower Neck: Unremarkable.

ABDOMEN AND PELVIS FINDINGS:

Liver: Unremarkable.

Gallbladder and Biliary Tree: Unremarkable.

Pancreas: Unremarkable.

Spleen: Unremarkable.

Adrenals: Unremarkable.

Kidneys and Ureters: Unremarkable.

Bladder: Unremarkable.

Reproductive Organs: Unremarkable.

Bowel: Unremarkable.

Lymph Nodes: No obvious lymphadenopathy.

Peritoneum: No free air, free fluid, or fluid collections.

Vessels: Unremarkable.

Abdominal wall: Unremarkable.

BONE FINDINGS: Non to minimally displaced fractures of the lateral/anterolateral left 3rd through 6th ribs

—

Dr. P Giuliano, MD - 07/01/2023

Formatting of this note might be different from the original.

CT CHEST, ABDOMEN, AND PELVIS WITHOUT CONTRAST

CLINICAL HISTORY: Fall. Left-sided chest and abdominal pain

COMPARISON: None.

TECHNIQUE: Using a multidetector helical CT scanner, thin slice acquisition was performed through the chest, abdomen, and pelvis without intravenous contrast. Multiplanar reformations were obtained. CT scan done according to ALARA.

DOSE INFORMATION:

CTDIvol (mGy)

Series 2 - Spiral - 12.84 (mGy)

Total DLP (mGy-cm): 885.50 mGy.cm.

(Note: The above reported CTDIvol and DLP values are CT scanner radiation output related dose indices, and, as such, they do NOT represent actual patient dose estimates. A medical physicist should be consulted for specific questions regarding the radiation dose for this exam).

Note that evaluation of the solid organs, soft tissues, and vascular structures is limited without intravenous contrast.

CHEST FINDINGS:

Airways: Unremarkable.

Lungs: 1.8 cm x 0.5 cm infiltrate in the right lower lobe posterior to the heart on image 107 series 3. Small nodular infiltrate measuring just over centimeter in the left upper lobe on image 88 series 3 with adjacent bronchiectasis

Pleura: No effusion or thickening.

Heart and Pericardium: Heart size is normal. No pericardial effusion.

Coronary Artery Calcification: Absent.

Vessels: Unremarkable.

Mediastinum and Hila: Unremarkable.

Chest Wall and Lower Neck: Unremarkable.

ABDOMEN AND PELVIS FINDINGS:

Liver: Unremarkable.

Gallbladder and Biliary Tree: Unremarkable.

Pancreas: Unremarkable.

Spleen: Unremarkable.

Adrenals: Unremarkable.

Kidneys and Ureters: Unremarkable.

Bladder: Unremarkable.

Reproductive Organs: Unremarkable.

Bowel: Unremarkable.

Lymph Nodes: No obvious lymphadenopathy.

Peritoneum: No free air, free fluid, or fluid collections.

Procedure Note

Vessels: Unremarkable.

Abdominal Wall: Unremarkable.

BONE FINDINGS: Non to minimally displaced fractures of the lateral/anterolateral left 3rd through 6th ribs

IMPRESSION:

- 1. Non to mildly displaced fractures of the left 3rd through 6th ribs without evidence for pneumothorax.
- 2. 1.2 cm peribronchial density in the left upper lobe probably infectious/inflammatory given associated bronchiectasis. 1.8 cm peri-fissural density in the medial right lower lobe. Recommend follow-up CT chest.

Electronically signed by Peter Giuliano 3/24/2023 8:06 AM

Authorizing Provider	Result Type	Result Status
B Felter NP	IMG CT PROCEDURES	Final Result

XR SHOULDER LEFT 2 PLUS VW - Final result (03/24/2023 1:27 AM PDT)

Anatomical Region	Laterality	Modality
Upper Extremities, Shoulder	Left	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:44 AM PDT

1. Multiple minimally displaced left-sided rib fractures. No significant pneumothorax No acute osseous abnormality of the left shoulder.

Electronically signed by Peter Giuliano 3/24/2023 7:44 AM

Narrative

03/24/2023 7:44 AM PDT

LEFT SHOULDER, 4 VIEWS

CLINICAL HISTORY: Fall. Pain. COMPARISON: None.

FINDINGS:

Several nondisplaced fractures of the lateral left rib cage noted approximately at the level of the 5th through 7th ribs No other acute fracture or dislocation. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

Procedure Note

Dr. P Giuliano, MD - 07/01/2023

Formatting of this note might be different from the original.

LEFT SHOULDER, 4 VIEWS

CLINICAL HISTORY: Fall. Pain. COMPARISON: None.

FINDINGS:

Several nondisplaced fractures of the lateral left rib cage noted approximately at the level of the 5th through 7th ribs No other acute fracture or dislocation. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

IMPRESSION:

1. Multiple minimally displaced left-sided rib fractures. No significant pneumothorax No acute osseous abnormality of the left shoulder.

Electronically signed by Peter Giuliano 3/24/2023 7:44 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

X-Ray (XR) Ankle Right 3 or more views (XR ANKLE 3+ VW RIGHT) - Final result (03/24/2023 1:27 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Right	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:44 AM PDT

1. No acute osseous abnormality of the right ankle.

Electronically signed by Shaya Ghazinoor 3/24/2023 7:44 AM

Narrative

03/24/2023 7:44 AM PDT

RIGHT ANKLE, 3 VIEWS

CLINICAL HISTORY: Ankle pain status post fall.

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The ankle mortise is symmetric and the other joint spaces are preserved. No significant soft tissue swelling.

Formatting of this note might be different from the original.

RIGHT ANKLE, 3 VIEWS

CLINICAL HISTORY: Ankle pain status post fall.

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The ankle mortise is symmetric and the other joint spaces are preserved. No significant soft tissue swelling.

IMPRESSION:

1. No acute osseous abnormality of the right ankle.

Electronically signed by Shaya Ghazinoor 3/24/2023 7:44 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

X-Ray (XR) Tibia Fibula Right 2 views (XR TIBIA FIBULA 2 VIEWS RIGHT) - Final result (03/24/2023 1:26 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Lower Leg	Right	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:42 AM PDT

1. No acute osseous abnormality of the right tibia and fibula.

Electronically signed by Peter Giuliano 3/24/2023 7:42 AM

Narrative

03/24/2023 7:42 AM PDT

RIGHT TIBIA AND FIBULA, 2 VIEWS

CLINICAL HISTORY: Pain midshaft; fall off bike with pain

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The joint spaces are preserved. No significant soft tissue swelling. No radiopaque foreign body

Formatting of this note might be different from the original.

RIGHT TIBIA AND FIBULA, 2 VIEWS

CLINICAL HISTORY: Pain midshaft; fall off bike with pain

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The joint spaces are preserved. No significant soft tissue swelling. No radiopaque foreign body

IMPRESSION:

1. No acute osseous abnormality of the right tibia and fibula.

Electronically signed by Peter Giuliano 3/24/2023 7:42 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

Visit Diagnoses - as of 08/29/2026

Diagnosis	Start Date
Multiple fractures of ribs, left side, initial encounter for closed fracture	3/24/2023
Contusion of right lower leg, initial encounter	3/24/2023
Multiple fractures of ribs, left side, subsequent encounter for fracture with routine healing	3/28/2023
Pleural effusion, not elsewhere classified	3/28/2023
Disorder of teeth and supporting structures, unspecified	6/9/2023
Wound of left lower extremity, initial encounter	2/11/2024
Rib pain on left side	2/11/2024
Cellulitis of other specified site	2/11/2024
LUQ pain	8/29/2024
Closed fracture of proximal end of left humerus with routine healing, unspecified fracture morphology, subsequent encounter	8/29/2024
Other closed displaced fracture of proximal end of left humerus, initial encounter	8/29/2024

Care Teams - as of 08/29/2026

Team Member	Relationship	Specialty	Start Date	End Date
PCp Not Assigned NPI: 1100007594 One Hoag Drive Newport Beach, CA 92663 949-764-4624 (Work)	PCP - General		2/11/24	

Document Information

Primary Care Provider	Other Service Providers	Document Coverage Dates
Pcp Not Assigned (Feb. 11, 2024 - Present) NPI: 1100007594 949-764-4624 (Work) One Hoag Drive Newport Beach, CA 92663 HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		May 04, 1983 - Aug. 29, 2026
Custodian Organization		
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		
Legal Authenticator		
Director H		

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Reason for Visit

- Reason
- Med Refill

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
09/14/2024 3:39 PM PDT -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER	C Caputo, PA-C 1 Hoag Dr Newport Beach, CA 92658 949-764-8372 (Work) 949-764-8378 (Fax)	Discharge Disposition: Home or Self Care
09/14/2024 5:06 PM PDT		1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	Dr. C Goldsworthy, MD 1 Hoag Dr Newport Beach, CA 92658-6100 949-764-8372 (Work) 949-764-8378 (Fax)	

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date	Status
<u>HYDROcodone-acetaminophen (NORCO) 10-325 mg tablet</u>	<u>Take 0.5 tablets by mouth every 6 (six) hours if needed for moderate pain (4-6) for up to 12 doses</u>	6 tablet		09/14/2024		Active
<u>HYDROcodone-acetaminophen (Norco) 5-325 mg tablet</u>	<u>Take 1-2 tablets by mouth every 6 (six) hours if needed for severe pain (7-10)</u>	15 tablet		08/29/2024	09/14/2024	Discontinued
<u>HYDROcodone-acetaminophen (Norco) 5-325 mg tablet</u>	<u>Take 1 tablet by mouth every 4 (four) hours if needed (Pain)</u>	12 tablet		09/14/2024	09/14/2024	Discontinued
<u>HYDROcodone-acetaminophen (Norco) 5-325 mg tablet</u>	<u>Take 1 tablet by mouth every 4 (four) hours if needed (Pain)</u>	12 tablet		09/14/2024	09/14/2024	Discontinued

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
<u>Smoking Tobacco: Some Days</u>				
Housing Stability		Answer		Date Recorded
Housing Stability		Not on file		08/05/2023
Housing Stability		Not on file		08/05/2023
Interpersonal Safety		Answer		Date Recorded
Interpersonal Safety		0		08/05/2023
Interpersonal Safety		0		08/05/2023
Interpersonal Safety		0		08/05/2023
Interpersonal Safety		0		08/05/2023
Sex and Gender Information		Value		Date Recorded
<u>Sex Assigned at Birth</u>		<u>Male</u>		<u>01/22/2025 10:41 AM PST</u>
<u>Legal Sex</u>		<u>Male</u>		<u>05/31/2023 8:08 PM PDT</u>
<u>Gender Identity</u>		<u>Male</u>		<u>01/22/2025 10:41 AM PST</u>
<u>Sexual Orientation</u>		<u>Straight</u>		<u>01/22/2025 10:41 AM PST</u>

Last Filed Vital Signs - documented in this encounter

Vital signs on 09/14/2024

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	<u>132/81</u>	09/14/2024 4:46 PM PDT	
Pulse	<u>91</u>	09/14/2024 4:46 PM PDT	
Temperature	<u>36.8 °C (98.3 °F)</u>	09/14/2024 4:46 PM PDT	
Respiratory Rate	<u>20</u>	09/14/2024 4:46 PM PDT	
Oxygen Saturation	<u>98%</u>	09/14/2024 4:46 PM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	<u>85.7 kg (188 lb 15 oz)</u>	09/14/2024 3:10 PM PDT	
Height	<u>182.9 cm (6')</u>	09/14/2024 3:10 PM PDT	
Body Mass Index	25.62	09/14/2024 3:10 PM PDT	

Discharge Instructions - documented in this encounter

Attachments

The following attachments were made available to the patient upon discharge.

Humerus Fracture Rehab (English)

Ordered Prescriptions - documented in this encounter

Prescription	Sig	Dispense Quantity	Last Filled	Start Date	End Date
<u>HYDROcodone-acetaminophen (NORCO) 10-325 mg tablet</u>	<u>Take 0.5 tablets by mouth every 6 (six) hours if needed for moderate pain (4-6) for up to 12 doses</u>	6 tablet		09/14/2024	
<u>HYDROcodone-acetaminophen (Norco) 5-325 mg tablet</u>	<u>Take 1 tablet by mouth every 4 (four) hours if needed (Pain)</u>	12 tablet		09/14/2024	09/14/2024
<u>HYDROcodone-acetaminophen (Norco) 5-325 mg tablet</u>	<u>Take 1 tablet by mouth every 4 (four) hours if needed (Pain)</u>	12 tablet		09/14/2024	09/14/2024

ED Notes - documented in this encounter

Nurse Jose F - 09/14/2024 3:18 PM PDT

Formatting of this note might be different from the original.
Pt. Came in for medication refill. Pt. Is unable to recall name of medication at this time, but states to have been a pain killer. Pt.s left arm is seen in a sling.

Electronically signed by Nurse Jose F at 09/14/2024 3:19 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
<u>Lipid (cholesterol) test</u>	05/04/1983		
<u>COVID-19 Vaccine (#1)</u>	11/04/1983		
<u>DTaP/Tdap/Td Vaccines (1 - Tdap)</u>	05/04/2002		
<u>Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)</u>	05/04/2002		
<u>Pneumococcal Vaccination (1 of 2 - PCV)</u>	05/04/2002		
<u>Influenza Vaccine (#1)</u>	09/01/2026		
HIB Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
HPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Hepatitis A Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
IPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningococcal B Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

X-Ray (XR) Left Shoulder 2 or More Views (XR SHOULDER LEFT 2+ VW) - Final result (09/14/2024 4:06 PM PDT)

Anatomical Region	Laterality	Modality		
<u>Shoulder</u>	<u>Left</u>	<u>Digital Radiography</u>		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			<u>09/14/2024 4:08 PM PDT</u>	

Impressions

09/14/2024 4:10 PM PDT

1. Redemonstrated comminuted fracture of the left humeral neck.

Electronically signed by January Lopez 9/14/2024 4:10 PM

Narrative

09/14/2024 4:10 PM PDT

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT SHOULDER, 4 views

CLINICAL HISTORY: Left shoulder pain after trauma.

COMPARISON: August 29, 2024.

FINDINGS:

There is redemonstration of a comminuted fracture through the left humeral neck with slight lateral displacement of the shaft relative to the head. There is evidence of hemarthrosis with widening of the subacromial space. The acromioclavicular joint is intact.

—

Procedure Note

Dr. J Lopez, MD - 09/14/2024

Formatting of this note might be different from the original.

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT SHOULDER, 4 views

CLINICAL HISTORY: Left shoulder pain after trauma.

COMPARISON: August 29, 2024.

FINDINGS:

There is redemonstration of a comminuted fracture through the left humeral neck with slight lateral displacement of the shaft relative to the head. There is evidence of hemarthrosis with widening of the subacromial space. The acromioclavicular joint is intact.

—

IMPRESSION:

1. Redemonstrated comminuted fracture of the left humeral neck.

Electronically signed by January Lopez 9/14/2024 4:10 PM

Authorizing Provider	Result Type	Result Status
C Caputo PA-C	IMG XR PROCEDURES	Final Result

Visit Diagnoses - documented in this encounter

Diagnosis

Closed fracture of proximal end of left humerus with routine healing, unspecified fracture morphology, subsequent encounter - Primary

Insurance - documented as of this encounter

Guarantor: Dimarcello, Anthony

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	05/04/1983	949-350-2312 (Home)	412 Olive Ave Huntington Beach, CA 92648

CALOPTIMA MONARCH MG

Member	Subscriber	Plan / Payer (Effective 11/16/2021-Present)
Name: Dimarcello, Anthony Member ID: 99451018G Relation to Subscriber: Self	Name: Dimarcello, Anthony Subscriber ID: 99451018G	Payer ID: CALOP Group ID: Not on file Type: Managed Care Medi-Cal Address: 9 TECHNOLOGY DRIVE Irvine, CA 92618

Care Teams - documented as of this encounter

Team Member	Relationship	Specialty	Start Date	End Date
Pcp Not Assigned NPI: 1100007594 One Hoag Drive Newport Beach, CA 92663 949-764-4624 (Work)	PCP - General		2/11/24	

Document Information

Primary Care Provider	Other Service Providers	Document Coverage Dates
Pcp Not Assigned (Feb. 11, 2024 - Present) NPI: 1100007594 949-764-4624 (Work) One Hoag Drive Newport Beach, CA 92663 HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		Sep. 14, 2024
Custodian Organization		
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		
Encounter Providers		Encounter Date
C Caputo, PA-C (Attending) NPI: 1366771891 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658 Primary Care	Dr. C Goldsworthy, MD (Admitting) NPI: 1932162773 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658-6100 Emergency Medicine	Sep. 14, 2024
Legal Authenticator		
Director H		

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Reason for Visit

Reason
Shortness of Breath

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
08/29/2024 9:55 PM PDT -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER	M Willenborg, PA-C 1 Hoag Dr Newport Beach, CA 92658 949-764-8372 (Work) 949-764-8378 (Fax)	Discharge Disposition: AMA
08/29/2024 10:56 PM PDT		1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	Dr. D Ritter, MD 1 Hoag Dr Newport Beach, CA 92658 949-764-8372 (Work) 949-764-8378 (Fax)	

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date	Status
HYDROcodone-acetaminophen (Norco) 5-325 mg tablet	Take 1-2 tablets by mouth every 6 (six) hours if needed for severe pain (7-10)	15 tablet		08/29/2024	09/14/2024	Discontinued

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Some Days				
Housing Stability	Answer		Date Recorded	
Housing Stability	Not on file		08/05/2023	
Housing Stability	Not on file		08/05/2023	
Interpersonal Safety	Answer		Date Recorded	
Interpersonal Safety	0		08/05/2023	
Interpersonal Safety	0		08/05/2023	
Interpersonal Safety	0		08/05/2023	
Interpersonal Safety	0		08/05/2023	
Sex and Gender Information	Value		Date Recorded	
Sex Assigned at Birth	Male		01/22/2025 10:41 AM PST	
Legal Sex	Male		05/31/2023 8:08 PM PDT	
Gender Identity	Male		01/22/2025 10:41 AM PST	
Sexual Orientation	Straight		01/22/2025 10:41 AM PST	

Last Filed Vital Signs - documented in this encounter

Vital signs on 08/29/2024

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	139/90	08/29/2024 8:31 PM PDT	
Pulse	105	08/29/2024 8:31 PM PDT	
Temperature	37 °C (98.6 °F)	08/29/2024 8:31 PM PDT	
Respiratory Rate	17	08/29/2024 8:31 PM PDT	
Oxygen Saturation	97%	08/29/2024 8:31 PM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	90.7 kg (200 lb)	08/29/2024 8:31 PM PDT	
Height	182.9 cm (6')	08/29/2024 8:31 PM PDT	
Body Mass Index	27.12	08/29/2024 8:31 PM PDT	

Discharge Instructions - documented in this encounter

Attachments

The following attachments were made available to the patient upon discharge.

Abdominal Pain Adult (English)

Chest Wall Pain (English)

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date
HYDROcodone-acetaminophen (Norco) 5-325 mg tablet	Take 1-2 tablets by mouth every 6 (six) hours if needed for severe pain (7-10)	15 tablet		08/29/2024	09/14/2024

ED Notes - documented in this encounter

Nurse Sakol C - 08/29/2024 8:28 PM PDT

Formatting of this note might be different from the original.

Pt co of SOB and L rib pain after falling off a skateboard yesterday. Pt stated that he broke his shoulder yesterday. Pt stated that he broke his ribs a year ago and his having worsening pain near ribs.

Electronically signed by Nurse Sakol C at 08/29/2024 8:31 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
<u>Lipid (cholesterol) test</u>	05/04/1983		
<u>COVID-19 Vaccine (#1)</u>	11/04/1983		
<u>DTaP/Tdap/Td Vaccines (1 - Tdap)</u>	05/04/2002		
<u>Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)</u>	05/04/2002		
<u>Pneumococcal Vaccination (1 of 2 - PCV)</u>	05/04/2002		
<u>Influenza Vaccine (#1)</u>	09/01/2026		
HIB Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
HPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Hepatitis A Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
IPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningococcal B Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

X-Ray (XR) Chest 1 View (XR CHEST 1 VW) - Final result (08/29/2024 9:58 PM PDT)

Anatomical Region		Laterality	Modality	
<u>Chest</u>			<u>Digital Radiography</u>	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			<u>08/29/2024 10:12 PM</u> <u>PDT</u>	

Impressions

08/29/2024 10:12 PM PDT

1. No evidence of acute cardiopulmonary disease.

Electronically signed by John Lieu 8/29/2024 10:12 PM

Narrative

08/29/2024 10:12 PM PDT

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

CHEST, 1 VIEW

CLINICAL HISTORY: TRAUMA.

COMPARISON: Chest x-ray 2/11/2024

FINDINGS:

The lungs are clear. The heart size is within normal limits.

No acute osseous process.

Procedure Note

Dr. J Lieu, MD - 08/29/2024

Formatting of this note might be different from the original.

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

CHEST, 1 VIEW

CLINICAL HISTORY: TRAUMA.

COMPARISON: Chest x-ray 2/11/2024

FINDINGS:

The lungs are clear. The heart size is within normal limits.

No acute osseous process.

IMPRESSION:

1. No evidence of acute cardiopulmonary disease.

Electronically signed by John Lieu 8/29/2024 10:12 PM

Authorizing Provider	Result Type	Result Status
Dr. J Houpt MD	IMG XR PROCEDURES	Final Result

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature				
<u>VENTRICULAR RATE EKG</u>	<u>89</u>	<u>bpm</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>ATRIAL RATE</u>	<u>87</u>	<u>bpm</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>P-R INTERVAL</u>	<u>134</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>P WAVE AXIS</u>	<u>76</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>QRS DURATION</u>	<u>97</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>Q-T INTERVAL</u>	<u>373</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>Q-T INTERVAL (CORRECTED)</u>	<u>455</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>QRS AXIS</u>	<u>56</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>T AXIS</u>	<u>45</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>INTERPRETATION TEXT</u>	<u>- NORMAL</u> <u>ECG -</u> <u>Sinus</u> <u>rhythm</u> <u>when</u> <u>compared</u> <u>with ECG of</u> <u>11-Feb-2024</u> <u>10:07:33,</u> <u>Nonspecific</u> <u>significant</u> <u>change</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>INTERPRETATION TEXT</u>	<u>Electronically</u> <u>signed by:</u> <u>Gandhi,</u> <u>Radhakrishan</u> <u>29-Aug-2024</u> <u>22:10:05</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>					
<u>Specimen (Source)</u>	<u>Anatomical Location /</u> <u>Laterality</u>		<u>Collection Method /</u> <u>Volume</u>		<u>Collection Time</u>	<u>Received Time</u>				
					<u>08/29/2024 8:14 PM PDT</u>					
<u>Narrative</u>										
<u>This result has an attachment that is not available.</u>										
<u>-</u>										
<u>Authorizing Provider</u>	<u>Result Type</u>		<u>Result Status</u>							
<u>Dr. J Houpt MD</u>	<u>ECG ORDERABLES</u>		<u>Final Result</u>							
<u>Performing Organization</u>	<u>Address</u>		<u>City/State/ZIP Code</u>		<u>Phone Number</u>					
<u>HOAG CV IECG</u> <u>RESULTING</u>					<u>-</u>					

Visit Diagnoses - documented in this encounter

Diagnosis
Rib pain on left side - Primary
LUQ pain
Closed fracture of proximal end of left humerus with routine healing, unspecified fracture morphology, subsequent encounter

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
<u>HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 2 tablet</u> <u>2 tablet, Oral, Once, On Thu 8/29/24 at 2245, For 1 dose, HHN/HHI Oral Opioid Admin Instructions</u> <u>First dose must be the lowest dose, titrate to effective dose by repeat of the lowest dose every 60 minutes prn pain, may not exceed maximum dose ordered per interval. Use Pasero Sedation Scale. If another order exists for the same level of PRN pain, use this agent after 1st ordered therapy is ineffective., OR contact provider to notify of a duplicate PRN reason.</u>	Given	08/29/2024 10:44 PM PDT	2 tablets		

Insurance - documented as of this encounter

Guarantor: Dimarcello, Anthony

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	05/04/1983	949-350-2312 (Home)	412 Olive Ave Huntington Beach, CA 92648

CALOPTIMA MONARCH MG

Member	Subscriber	Plan / Payer (Effective 11/16/2021-Present)
<u>Name: Dimarcello, Anthony</u> <u>Member ID: 99451018G</u> <u>Relation to Subscriber: Self</u>	<u>Name: Dimarcello, Anthony</u> <u>Subscriber ID: 99451018G</u>	<u>Payer ID: CALOP</u> <u>Group ID: Not on file</u> <u>Type: Managed Care Medi-Cal</u> <u>Address:</u> <u>9 TECHNOLOGY DRIVE</u> <u>Irvine, CA 92618</u>

Care Teams - documented as of this encounter

Team Member	Relationship	Specialty	Start Date	End Date
<u>Pcp Not Assigned</u> <u>NPI: 1100007594</u> <u>One Hoag Drive</u> <u>Newport Beach, CA 92663</u> <u>949-764-4624 (Work)</u>	<u>PCP - General</u>		<u>2/11/24</u>	

Document Information

Primary Care Provider		Other Service Providers	Document Coverage Dates
Pcp Not Assigned (Feb. 11, 2024 - Present) NPI: 1100007594 949-764-4624 (Work) One Hoag Drive Newport Beach, CA 92663 HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658			Aug. 29, 2024
Custodian Organization			
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658			
Encounter Providers		Encounter Date	
M Willenborg, PA-C (Attending) NPI: 1598213886 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658 Primary Care	Dr. D Ritter, MD (Admitting) NPI: 1770004608 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658 Emergency Medicine	Aug. 29, 2024	
Legal Authenticator			
Director H			

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Reason for Visit

Reason
Fall
Shoulder Injury

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
08/29/2024 2:54 AM PDT -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER	Dr. E Kang, MD 1 Hoag Dr Newport Beach, CA 92658-6100 949-764-8372 (Work) 949-764-8378 (Fax)	Discharge Disposition: Home or Self Care
08/29/2024 6:28 AM PDT		1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372		

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date	Status
oxyCODONE-acetaminophen (PERCOCET) 5-325 mg tablet	Take 1 tablet by mouth every 6 (six) hours if needed for severe pain (7-10)	12 tablet		08/29/2024	08/29/2024	Discontinued
HYDROcodone-acetaminophen (Norco) 5-325 mg tablet	Take 1-2 tablets by mouth every 6 (six) hours if needed for severe pain (7-10)	15 tablet		08/29/2024	09/14/2024	Discontinued

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Some Days				
Housing Stability		Answer	Date Recorded	
Housing Stability		Not on file	08/05/2023	
Housing Stability		Not on file	08/05/2023	
Interpersonal Safety		Answer	Date Recorded	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Male	01/22/2025 10:41 AM PST	
Legal Sex		Male	05/31/2023 8:08 PM PDT	
Gender Identity		Male	01/22/2025 10:41 AM PST	
Sexual Orientation		Straight	01/22/2025 10:41 AM PST	

Last Filed Vital Signs - documented in this encounter

Vital signs on 08/29/2024

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	139/96	08/29/2024 6:00 AM PDT	
Pulse	84	08/29/2024 6:00 AM PDT	
Temperature	36.7 °C (98 °F)	08/29/2024 6:00 AM PDT	
Respiratory Rate	20	08/29/2024 6:00 AM PDT	
Oxygen Saturation	95%	08/29/2024 6:00 AM PDT	
Inhaled Oxygen Concentration	-	-	
Weight	90.2 kg (198 lb 13.7 oz)	08/29/2024 2:59 AM PDT	
Height	182.9 cm (6')	08/29/2024 2:59 AM PDT	
Body Mass Index	26.97	08/29/2024 2:59 AM PDT	

Discharge Instructions - documented in this encounter

Attachments

The following attachments were made available to the patient upon discharge.

Humerus Fracture Treated With Immobilization Easy-to-Read (English)

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date
HYDROcodone-acetaminophen (Norco) 5-325 mg tablet	Take 1-2 tablets by mouth every 6 (six) hours if needed for severe pain (7-10)	15 tablet		08/29/2024	09/14/2024

Ordered Prescriptions - documented in this encounter

Prescription	Sig	Dispense Quantity	Last Filled	Start Date	End Date
HYDROcodone-acetaminophen (Norco) 5-325 mg tablet	Take 1-2 tablets by mouth every 6 (six) hours if needed for severe pain (7-10)	15 tablet		08/29/2024	09/14/2024
oxyCODONE-acetaminophen (PERCOCET) 5-325 mg tablet	Take 1 tablet by mouth every 6 (six) hours if needed for severe pain (7-10)	12 tablet		08/29/2024	08/29/2024

ED Notes - documented in this encounter

Nurse Katrina P - 08/29/2024 2:56 AM PDT

Formatting of this note might be different from the original.

BIBA from beach. Pt was riding his skateboard and fell down and landed on the left shoulder. Denies LOC and head injury. Denies etoh, drug use. Denies blood thinners.

Electronically signed by Nurse Katrina P at 08/29/2024 2:58 AM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
<u>Lipid (cholesterol) test</u>	05/04/1983		
<u>COVID-19 Vaccine (#1)</u>	11/04/1983		
<u>DTaP/Tdap/Td Vaccines (1 - Tdap)</u>	05/04/2002		
<u>Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)</u>	05/04/2002		
<u>Pneumococcal Vaccination (1 of 2 - PCV)</u>	05/04/2002		
<u>Influenza Vaccine (#1)</u>	09/01/2026		
HIB Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
HPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Hepatitis A Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
IPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningococcal B Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

X-Ray (XR) Left Humerus 2 or More Views (XR HUMERUS LEFT 2+ VW) - Final result (08/29/2024 3:56 AM PDT)

Anatomical Region	Laterality	Modality		
<u>Upper Extremities</u>	<u>Left</u>	<u>Digital Radiography</u>		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			<u>08/29/2024 7:47 AM</u> <u>PDT</u>	

Impressions

08/29/2024 7:48 AM PDT

1. Acute mildly displaced proximal left humerus fracture.

Electronically signed by Gerard Nguyen 8/29/2024 7:48 AM

Narrative

08/29/2024 7:48 AM PDT

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT HUMERUS, 2 VIEWS

CLINICAL HISTORY: Left shoulder pain.

COMPARISON: None.

FINDINGS:

Acute proximal humerus neck fracture with lateral displacement approximately one half shaft width. The joint spaces are preserved. No significant soft tissue swelling.

-

Procedure Note

Dr. G Nguyen, MD - 08/29/2024

Formatting of this note might be different from the original.

Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO

LEFT HUMERUS, 2 VIEWS

CLINICAL HISTORY: Left shoulder pain.

COMPARISON: None.

FINDINGS:

Acute proximal humerus neck fracture with lateral displacement approximately one half shaft width. The joint spaces are preserved. No significant soft tissue swelling.

IMPRESSION:

1. Acute mildly displaced proximal left humerus fracture.

Electronically signed by Gerard Nguyen 8/29/2024 7:48 AM

Authorizing Provider	Result Type	Result Status
Dr. E Kang MD	IMG XR PROCEDURES	Final Result

X-Ray (XR) Left Shoulder 2 or More Views (XR SHOULDER LEFT 2+ VW) - Final result (08/29/2024 3:55 AM PDT)

Anatomical Region	Laterality	Modality
Shoulder	Left	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			08/29/2024 7:48 AM PDT	
Impressions				
08/29/2024 7:49 AM PDT				
1. Acute mildly displaced proximal left humerus fracture.				
Electronically signed by Gerard Nguyen 8/29/2024 7:49 AM				
Narrative				
08/29/2024 7:49 AM PDT				
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO				
XR SHOULDER LEFT 2+ VW				
CLINICAL HISTORY: Left shoulder pain.				
COMPARISON: None.				
FINDINGS:				
Acute comminuted proximal humerus neck fracture with lateral displacement approximately one half shaft width. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.				
-				
Procedure Note				
Dr. G Nguyen, MD - 08/29/2024				
Formatting of this note might be different from the original.				
Patient MRN: 5109648 Patient Name: ANTHONY DIMARCELLO				
XR SHOULDER LEFT 2+ VW				
CLINICAL HISTORY: Left shoulder pain.				
COMPARISON: None.				
FINDINGS:				
Acute comminuted proximal humerus neck fracture with lateral displacement approximately one half shaft width. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.				
IMPRESSION:				
1. Acute mildly displaced proximal left humerus fracture.				
Electronically signed by Gerard Nguyen 8/29/2024 7:49 AM				
Authorizing Provider	Result Type	Result Status		
Dr. E Kang MD	IMG XR PROCEDURES	Final Result		

Visit Diagnoses - documented in this encounter

Diagnosis

Other closed displaced fracture of proximal end of left humerus, initial encounter - Primary

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
<u>HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 2 tablet</u> <u>2 tablet, Oral, Once, On Thu 8/29/24 at 0330, For 1 dose, HHN/HHI Oral Opioid Admin Instructions</u> <u>First dose must be the lowest dose, titrate to effective dose by repeat of the lowest dose every 60 minutes prn pain, may not exceed maximum dose ordered per interval. Use Pasero Sedation Scale. If another order exists for the same level of PRN pain, use this agent after 1st ordered therapy is ineffective., OR contact provider to notify of a duplicate PRN reason.</u>	Given	08/29/2024 3:29 AM PDT	2 tablets		

Insurance - documented as of this encounter

Guarantor: Dimarcello, Anthony

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	05/04/1983	949-350-2312 (Home)	412 Olive Ave Huntington Beach, CA 92648

CALOPTIMA MONARCH MG

Member	Subscriber	Plan / Payer (Effective 11/16/2021-Present)
<u>Name: Dimarcello, Anthony</u> <u>Member ID: 99451018G</u> <u>Relation to Subscriber: Self</u>	<u>Name: Dimarcello, Anthony</u> <u>Subscriber ID: 99451018G</u>	<u>Payer ID: CALOP</u> <u>Group ID: Not on file</u> <u>Type: Managed Care Medi-Cal</u> <u>Address:</u> <u>9 TECHNOLOGY DRIVE</u> <u>Irvine, CA 92618</u>

Care Teams - documented as of this encounter

Team Member	Relationship	Specialty	Start Date	End Date
<u>PCp Not Assigned</u> <u>NPI: 1100007594</u> <u>One Hoag Drive</u> <u>Newport Beach, CA 92663</u> <u>949-764-4624 (Work)</u>	<u>PCP - General</u>		<u>2/11/24</u>	

Document Information

Primary Care Provider	Other Service Providers	Document Coverage Dates
Pcp Not Assigned (Feb. 11, 2024 - Present) NPI: 1100007594 949-764-4624 (Work) One Hoag Drive Newport Beach, CA 92663 HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		Aug. 29, 2024
Custodian Organization		
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		
Encounter Providers	Encounter Date	
Dr. E Kang, MD (Attending, Admitting) NPI: 1386931244 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658-6100 Emergency Medicine		Aug. 29, 2024
Legal Authenticator		
Director H		

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Reason for Visit

Reason
Hip Pain

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
02/11/2024 9:46 AM PST -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER	Dr. H Marino, DO 1 Hoag Dr Newport Beach, CA 92658 949-764-8372 (Work) 949-764-8378 (Fax)	Discharge Disposition: Home or Self Care
02/11/2024 12:58 PM PST		1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372		

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date	Status
cephalexin (Keflex) 500 mg capsule	Take 1 capsule (500 mg total) by mouth 4 (four) times a day for 7 days	28 capsule		02/11/2024	02/18/2024	Expired
sulfamethoxazole-trimethoprim (BACTRIM DS) 800-160 mg tablet	Take 1 tablet by mouth 2 (two) times a day for 7 days	14 tablet		02/11/2024	02/18/2024	Expired
ibuprofen (ADVIL,MOTRIN) 600 MG tablet	Take 1 tablet (600 mg total) by mouth every 6 (six) hours if needed for mild pain (1-3) for up to 10 days	30 tablet		02/11/2024	02/21/2024	Expired

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Some Days				
Housing Stability		Answer	Date Recorded	
Housing Stability		Not on file	08/05/2023	
Housing Stability		Not on file	08/05/2023	
Interpersonal Safety		Answer	Date Recorded	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Male	01/22/2025 10:41 AM PST	
Legal Sex		Male	05/31/2023 8:08 PM PDT	
Gender Identity		Male	01/22/2025 10:41 AM PST	
Sexual Orientation		Straight	01/22/2025 10:41 AM PST	

Last Filed Vital Signs - documented in this encounter

Vital signs on 02/11/2024

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	125/81	02/11/2024 12:52 PM PST	
Pulse	74	02/11/2024 12:52 PM PST	
Temperature	36.7 °C (98 °F)	02/11/2024 12:52 PM PST	
Respiratory Rate	16	02/11/2024 12:52 PM PST	
Oxygen Saturation	100%	02/11/2024 12:52 PM PST	
Inhaled Oxygen Concentration	-	-	
Weight	83.5 kg (184 lb 1.4 oz)	02/11/2024 10:02 AM PST	
Height	177.8 cm (5' 10")	02/11/2024 10:02 AM PST	
Body Mass Index	26.41	02/11/2024 10:02 AM PST	

Discharge Instructions - documented in this encounter

Attachments

The following attachments were made available to the patient upon discharge.

- Wound Care Adult (English)
- Wound Infection Easy-to-Read (English)
- Chest Wall Pain Easy-to-Read (English)

Medications at Time of Discharge - documented as of this encounter

Medication	Sig	Dispense Quantity	Last Filled	Start Date	End Date
cephalexin (Keflex) 500 mg capsule	Take 1 capsule (500 mg total) by mouth 4 (four) times a day for 7 days	28 capsule		02/11/2024	02/18/2024
ibuprofen (ADVIL,MOTRIN) 600 MG tablet	Take 1 tablet (600 mg total) by mouth every 6 (six) hours if needed for mild pain (1-3) for up to 10 days	30 tablet		02/11/2024	02/21/2024
sulfamethoxazole-trimethoprim (BACTRIM DS) 800-160 mg tablet	Take 1 tablet by mouth 2 (two) times a day for 7 days	14 tablet		02/11/2024	02/18/2024

Ordered Prescriptions - documented in this encounter

Prescription	Sig	Dispense Quantity	Last Filled	Start Date	End Date
<u>ibuprofen (ADVIL,MOTRIN) 600 MG tablet</u>	<u>Take 1 tablet (600 mg total) by mouth every 6 (six) hours if needed for mild pain (1-3) for up to 10 days</u>	30 tablet		02/11/2024	02/21/2024
<u>sulfamethoxazole-trimethoprim (BACTRIM DS) 800-160 mg tablet</u>	<u>Take 1 tablet by mouth 2 (two) times a day for 7 days</u>	14 tablet		02/11/2024	02/18/2024
<u>cephalexin (Keflex) 500 mg capsule</u>	<u>Take 1 capsule (500 mg total) by mouth 4 (four) times a day for 7 days</u>	28 capsule		02/11/2024	02/18/2024

ED Notes - documented in this encounter

Nurse Carolina G - 02/11/2024 9:50 AM PST

Formatting of this note might be different from the original.

Pt biba from front of HBPD for left hip pain x 1 week. Pt reports staph infection from wound on his left hip. Hx of rib fx. NKA. No meds.

Electronically signed by Nurse Carolina G at 02/11/2024 9:51 AM PST

Electronically signed by Nurse Carolina G at 02/11/2024 9:52 AM PST

Miscellaneous Notes - documented in this encounter

Care Plan - Gulhan R - 02/11/2024 12:55 PM PST

Formatting of this note might be different from the original.

ED SW Consult Note:

Per chart, patient is a 40 y.o. male who was BIBA and presents with hip pain x 1 week. Patient reports left hip pain and wound that has been gradually worsening since initial onset. Patient seeking assistance with housing. SW met with patient at bedside to introduce self/role and assess needs. Patient shared that he has been trying to get into Mercy House shelter. Discussed options to call Illumination Foundation and CityNet in addition to the other shelters given; encouraged the pt to continue to follow with Mercy House. Provided resources for Legal Aid per requests due to "My identity ws stole." Pt denied further needs and thanked SW. RN also at bedside.

SW remains available.

Electronically signed by Gulhan R at 02/11/2024 12:59 PM PST

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
<u>Lipid (cholesterol) test</u>	05/04/1983		
<u>COVID-19 Vaccine (#1)</u>	11/04/1983		
<u>DTaP/Tdap/Td Vaccines (1 - Tdap)</u>	05/04/2002		
<u>Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)</u>	05/04/2002		
<u>Pneumococcal Vaccination (1 of 2 - PCV)</u>	05/04/2002		
<u>Influenza Vaccine (#1)</u>	09/01/2026		
HIB Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
HPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Hepatitis A Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
IPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningococcal B Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

TROPONIN I - Final result (02/11/2024 10:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Troponin I</u>	<u><0.01</u>	<u>0.00 - 0.03 ng/mL</u>		<u>02/11/2024 10:58 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
<u>Blood</u>	<u>Venous blood specimen / Unknown</u>	<u>Venipuncture / Unknown</u>		<u>02/11/2024 10:19 AM PST</u>	<u>02/11/2024 10:24 AM PST</u>	
Narrative						
-						
Authorizing Provider	Result Type	Result Status				
<u>Dr. H Marino DO</u>	<u>LAB BLOOD ORDERABLES</u>	<u>Final Result</u>				
Performing Organization	Address	City/State/ZIP Code	Phone Number			
<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	<u>One Hoag Drive</u>	<u>Newport Beach, CA 92663, US</u>	<u>949-764-5600</u>			

COMPREHENSIVE METABOLIC PANEL - Final result (02/11/2024 10:19 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Sodium (Na)</u>	<u>139</u>	<u>135 - 145 mmol/L</u>		<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Potassium (K)</u>	<u>4.0</u>	<u>3.5 - 5.2 mmol/L</u>		<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Chloride (Cl)</u>	<u>106</u>	<u>100 - 110 mmol/L</u>		<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Carbon Dioxide (CO2)</u>	<u>27</u>	<u>24 - 32</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Anion Gap</u>	<u>6</u>	<u>6 - 14</u> <u>mmol/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Glucose</u>	<u>97</u>	<u>65 - 99</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>BUN (Blood Urea Nitrogen)</u>	<u>17</u>	<u>9 - 20</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Creatinine</u>	<u>0.70</u>	<u>0.40 -</u> <u>1.50 mg/</u> <u>dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>eGFR (Estimated Glomerular Filtration Rate)</u>	<u>>60</u>	<u>>=60</u> <u>mL/min/</u> <u>1.73m2</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Comment:

GFR value was calculated using a new GFR equation effective since 1/18/22. This is a recommendation of the American Society of Nephrology and National Kidney Foundation.

Clinical practice guidelines suggest the use of serum cystatin C as a confirmatory test for eGFR 45-59 ml/min/1.73m2 in adults who do not have markers of kidney disease; eGFR may be less accurate in this range.

<u>Calcium (Ca)</u>	<u>8.7</u>	<u>8.4 - 10.2</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
---------------------	------------	-----------------------------------	--	--	--	--

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Albumin</u>	<u>4.0</u>	<u>3.5 - 5.0</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Bilirubin, Total</u>	<u>0.5</u>	<u>0.2 - 1.3</u> <u>mg/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Total Protein</u>	<u>6.6</u>	<u>6.3 - 8.2</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>AST</u>	<u>32</u>	<u>17 - 59</u> <u>U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>ALT (Alanine Aminotransferase)(SGPT)</u>	<u>34</u>	<u><50 U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Alkaline Phosphatase</u>	<u>64</u>	<u>38 - 126</u> <u>U/L</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Globulin</u>	<u>2.6</u>	<u>2.0 - 4.0</u> <u>g/dL</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Albumin/Globulin Ratio</u>	<u>1.5</u>	<u>1.2 - 2.0</u>		<u>02/11/2024</u> <u>10:48 AM</u> <u>PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>BUN/Creatinine Ratio</u>	<u>24.3</u>			<u>02/11/2024 10:48 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume	Collection Time	Received Time	
<u>Blood</u>	<u>Venous blood specimen / Unknown</u>		<u>Venipuncture / Unknown</u>	<u>02/11/2024 10:19 AM PST</u>	<u>02/11/2024 10:24 AM PST</u>	
Narrative						
—						
Authorizing Provider	Result Type		Result Status			
<u>Dr. H Marino DO</u>	<u>LAB BLOOD ORDERABLES</u>		<u>Final Result</u>			
Performing Organization	Address		City/State/ZIP Code	Phone Number		
<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	<u>One Hoag Drive</u>		<u>Newport Beach, CA 92663, US</u>	<u>949-764-5600</u>		

CBC WITH DIFFERENTIAL (CBC AND DIFFERENTIAL - WAM AND NON-WAM) - Final result (02/11/2024 10:19 AM PST)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>WBC (White Blood Cells)</u>	<u>6.78</u>	<u>4.80 - 11.00 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Red Blood Cells</u>	<u>4.77</u>	<u>4.20 - 6.20 M/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Hemoglobin</u>	<u>14.4</u>	<u>14.0 - 18.0 g/dL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Hematocrit</u>	<u>43.7</u>	<u>42.0 - 54.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>MCV</u>	<u>91.6</u>	<u>80.0 - 100.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>MCH</u>	<u>30.2</u>	<u>27.0 - 31.0 pg</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>MCHC</u>	<u>33.0</u>	<u>32.0 - 37.0 g/dL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Platelet Count</u>	<u>238</u>	<u>150 - 400 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>RDW-SD</u>	<u>40.1</u>	<u>36.0 - 55.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>RDWCV</u>	<u>11.9</u>	<u>11.5 - 14.5 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>MPV</u>	<u>9.3</u>	<u>9.0 - 13.0 fL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% nRBC</u>	<u>0</u>	<u><1 per 100 WBCs</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>% Neutrophils</u>	<u>65.0</u>	<u>36.0 - 75.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Lymphocytes</u>	<u>26.4</u>	<u>13.0 - 50.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Monocytes</u>	<u>6.6</u>	<u>2.0 - 10.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Eosinophils</u>	<u>1.3</u>	<u>0.0 - 6.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Basophils</u>	<u>0.4</u>	<u>0.0 - 2.0 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>% Immature Granulocytes</u>	<u>0.3</u>	<u>0.0 - 0.8 %</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Neutrophils</u>	<u>4.40</u>	<u>1.00 - 7.00 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	
<u>Absolute Lymphocytes</u>	<u>1.79</u>	<u>1.50 - 4.00 K/ uL</u>		<u>02/11/2024 10:32 AM PST</u>	HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>Absolute Monocytes</u>	<u>0.45</u>	<u>0.20 - 0.90 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Absolute Eosinophils</u>	<u>0.09</u>	<u>0.00 - 0.70 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Absolute Basophils</u>	<u>0.03</u>	<u>0.00 - 0.20 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Absolute Immature Granulocytes</u>	<u>0.02</u>	<u>0.00 - 0.10 K/uL</u>		<u>02/11/2024 10:32 AM PST</u>	<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	
<u>Specimen (Source)</u>	<u>Anatomical Location / Laterality</u>		<u>Collection Method / Volume</u>		<u>Collection Time</u>	<u>Received Time</u>
<u>Blood</u>	<u>Venous blood specimen / Unknown</u>		<u>Venipuncture / Unknown</u>		<u>02/11/2024 10:19 AM PST</u>	<u>02/11/2024 10:24 AM PST</u>
<u>Narrative</u>						
—						
<u>Authorizing Provider</u>	<u>Result Type</u>		<u>Result Status</u>			
<u>Dr. H Marino DO</u>	<u>LAB BLOOD ORDERABLES</u>		<u>Final Result</u>			
<u>Performing Organization</u>	<u>Address</u>		<u>City/State/ZIP Code</u>	<u>Phone Number</u>		
<u>HOAG MEMORIAL HOSPITAL NEWPORT BEACH LABORATORY (CLIA 05D0578635)</u>	<u>One Hoag Drive</u>		<u>Newport Beach, CA 92663, US</u>	<u>949-764-5600</u>		
<u>12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (02/11/2024 10:07 AM PST)</u>						
<u>Component</u>	<u>Value</u>	<u>Ref Range</u>	<u>Test Method</u>	<u>Analysis Time</u>	<u>Performed At</u>	<u>Pathologist Signature</u>
<u>VENTRICULAR RATE EKG</u>	<u>72</u>	<u>bpm</u>			<u>HOAG CV IECG RESULTING</u>	
<u>ATRIAL RATE</u>	<u>71</u>	<u>bpm</u>			<u>HOAG CV IECG RESULTING</u>	
<u>P-R INTERVAL</u>	<u>167</u>	<u>ms</u>			<u>HOAG CV IECG RESULTING</u>	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>P WAVE AXIS</u>	<u>50</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>QRS DURATION</u>	<u>98</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Q-T INTERVAL</u>	<u>446</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>488</u>	<u>ms</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>QRS AXIS</u>	<u>63</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>T AXIS</u>	<u>58</u>	<u>deg</u>			<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>INTERPRETATION TEXT</u>	<u>-</u> <u>BORDERLINE</u> <u>ECG -</u> <u>Sinus</u> <u>rhythm</u> <u>Borderline</u> <u>prolonged</u> <u>QT interval</u> <u>when</u> <u>compared</u> <u>with ECG of</u> <u>28-Mar-2023</u> <u>17:35:28,</u> <u>Significant</u> <u>rate</u> <u>decrease</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>INTERPRETATION TEXT</u>	<u>Electronically</u> <u>signed by:</u> <u>Kuo, Allen 11-</u> <u>Feb-2024</u> <u>19:45:41</u>				<u>HOAG CV</u> <u>IECG</u> <u>RESULTING</u>	
<u>Specimen (Source)</u>	<u>Anatomical Location /</u> <u>Laterality</u>	<u>Collection Method /</u> <u>Volume</u>		<u>Collection Time</u>	<u>Received Time</u>	
				<u>02/11/2024 10:07 AM</u> <u>PST</u>		
<u>Narrative</u>						
<u>This result has an attachment that is not available.</u>						
<u>-</u>						
<u>Authorizing Provider</u>	<u>Result Type</u>	<u>Result Status</u>				
<u>Dr. H Marino DO</u>	<u>ECG ORDERABLES</u>	<u>Final Result</u>				
<u>Performing Organization</u>	<u>Address</u>	<u>City/State/ZIP Code</u>	<u>Phone Number</u>			
<u>HOAG CV IECG</u> <u>RESULTING</u>			<u>-</u>			
<u>X-Ray (XR) Chest Anterior/Posterior Portable (XR CHEST AP PORTABLE) - Final result (02/11/2024 10:05 AM PST)</u>						
<u>Anatomical Region</u>	<u>Laterality</u>	<u>Modality</u>				
<u>Chest</u>		<u>Digital Radiography</u>				

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			02/11/2024 10:13 AM PST	
Impressions				
02/11/2024 10:14 AM PST				
1. No evidence of acute cardiopulmonary disease.				
Electronically signed by Humberto wong 2/11/2024 10:14 AM				
Narrative				
02/11/2024 10:14 AM PST				
CHEST 1 VIEW 2/11/2024 9:59 AM				
CLINICAL HISTORY: Pain				
COMPARISON: 6/9/2023				
FINDINGS:				
Lungs are clear. Cardiomediastinal silhouette is normal. No pleural effusions or pneumothorax. No acute bony abnormalities				
Procedure Note				
Dr. H Wong, MD - 02/11/2024				
Formatting of this note might be different from the original.				
CHEST 1 VIEW 2/11/2024 9:59 AM				
CLINICAL HISTORY: Pain				
COMPARISON: 6/9/2023				
FINDINGS:				
Lungs are clear. Cardiomediastinal silhouette is normal. No pleural effusions or pneumothorax. No acute bony abnormalities				
IMPRESSION:				
1. No evidence of acute cardiopulmonary disease.				
Electronically signed by Humberto Wong 2/11/2024 10:14 AM				
Authorizing Provider	Result Type	Result Status		
Dr. H Marino DO	IMG XR PROCEDURES	Final Result		

Visit Diagnoses - documented in this encounter

Diagnosis

Wound of left lower extremity, initial encounter - Primary

Rib pain on left side

Cellulitis of other specified site

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
ibuprofen (ADVIL, MOTRIN) tablet 600 mg 600 mg, Oral, Once, On Sun 2/11/24 at 1000, For 1 dose, Give with food.	Given	02/11/2024 10:22 AM PST	600 mg		

Insurance - documented as of this encounter

Guarantor: Dimarcello, Anthony

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	05/04/1983	949-350-2312 (Home)	412 Olive Ave Huntington Beach, CA 92648

CALOPTIMA MONARCH MG

Member	Subscriber	Plan / Payer (Effective 11/16/2021-Present)
Name: Dimarcello, Anthony Member ID: 99451018G Relation to Subscriber: Self	Name: Dimarcello, Anthony Subscriber ID: 99451018G	Payer ID: CALOP Group ID: Not on file Type: Managed Care Medi-Cal Address: 9 TECHNOLOGY DRIVE Irvine, CA 92618

Care Teams - documented as of this encounter

Team Member	Relationship	Specialty	Start Date	End Date
Pcp Not Assigned NPI: 1100007594 One Hoag Drive Newport Beach, CA 92663 949-764-4624 (Work)	PCP - General		2/11/24	

Document Information

Primary Care Provider	Other Service Providers	Document Coverage Dates
Pcp Not Assigned (Feb. 11, 2024 - Present) NPI: 1100007594 949-764-4624 (Work) One Hoag Drive Newport Beach, CA 92663 HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		Feb. 11, 2024
Custodian Organization		
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658		
Encounter Providers		Encounter Date
Dr. H Marino, DO (Attending, Admitting) NPI: 1619273018 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658 Emergency Medicine		Feb. 11, 2024
Legal Authenticator		
Director H		

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
06/09/2023 6:52 PM PDT -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER	Dr. E Alcouloumre, MD <u>1 Hoag Dr</u> <u>Newport Beach, CA 92658</u> <u>949-764-8372 (Work)</u> <u>949-764-8378 (Fax)</u>	<u>Discharge Disposition: Home or</u> <u>Self Care</u>
06/09/2023 8:32 PM PDT		1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372		

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

No known medications

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Some Days				
Housing Stability		Answer	Date Recorded	
Housing Stability		Not on file	08/05/2023	
Housing Stability		Not on file	08/05/2023	
Interpersonal Safety		Answer	Date Recorded	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Male	01/22/2025 10:41 AM PST	
Legal Sex		Male	05/31/2023 8:08 PM PDT	
Gender Identity		Male	01/22/2025 10:41 AM PST	
Sexual Orientation		Straight	01/22/2025 10:41 AM PST	

Last Filed Vital Signs - documented in this encounter

Not on file

Discharge Instructions - documented in this encounter

Discharge Instructions
PROVIDER, HOAG IP CONVERSION - 06/09/2023 7:39 PM PDT
Formatting of this note might be different from the original.
Follow up at SOS clinic to establish primary care for referral to cardiothoracic surgery regarding your chronic rib fractures
Follow up with dentist outpatient
Electronically signed by K Claunch, PA at 06/09/2023 8:00 PM PDT
Electronically signed by PROVIDER, HOAG IP CONVERSION at 07/27/2023 11:44 PM PDT

ED Notes - documented in this encounter

K Claunch, PA - 06/09/2023 7:00 PM PDT

Formatting of this note might be different from the original.

EMERGENCY DEPARTMENT HISTORY AND PHYSICAL
HOAG HOSPITAL PRESBYTERIAN

6/9/2023
7:00 PM PDT

Patient Name: Anthony Dimarcello **DOB**: 5/4/1983 **Medical Record**: 20012489767
PCP: None disclosed

My Supervising physician is Dr. Alcouloumre, Eric M., M.D.

CHIEF COMPLAINT

[table#1]:
[**Chief Complaint**]
[---]
[Patient presents with]
[• Rib Pain]
[• Medication Refill]

HPI

Anthony Dimarcello is a 40 y.o. male who presents with rib pain onset 2 weeks ago. The patient was seen here at HHNB on 3/24/2023 for a closed fracture of multiple ribs on his left side due to an E-bike accident where he eloped from the ED. He then returned to the ED on 3/28/23 here at HHNB for SOB where he was also diagnosed with pleural effusion of the left side along with his rib fractures and discharged with a plan to follow up with Nguyen M. Le, MD (Thoracic Surgery). The patient states that his pain today is from this incident but that 2 weeks ago his pain has been increasing. He claims that he has been lifting heavy recently and participating in extraneous activity. He notes associated left dental pain due to impacted wisdom teeth which he feels was made worse by the accident, but denies any falls since the primary incident or recent trauma. He has been taking ibuprofen for pain but states that recently it is no longer providing him with relief.

History Obtained from Independent historian: None

REVIEW OF SYSTEMS

See HPI for further details. Review of systems otherwise negative.

PAST MEDICAL HISTORY

History reviewed. No pertinent past medical history.

SURGICAL HISTORY

History reviewed. No pertinent surgical history.

SOCIAL HISTORY

[table#2]:

Social History
|---|

[table#3]:

Tobacco Use
|---|

|•| Smoking status: | Some Days |

||| Types: | Cigarettes |

Substance Use Topics

|•| Alcohol use: | Yes |

||| *Comment: sometimes* |

|•| Drug use: | Yes |

||| Types: | Marijuana |

||| *Comment: sometimes* |

CURRENT MEDICATIONS

[table#4]:

No current outpatient medications on file prior to encounter.
|---|

ALLERGIES

No Known Allergies

PHYSICAL EXAM

VITAL SIGNS: Temp: 36.4 °C (97.5 °F) Pulse: 89 Resp: 18 BP: 151/63 SpO2: 96 %

GENERAL: WDOWN, NAD

HEENT: NCAT

NECK: normal ROM

CVS: regular rate and rhythm.

RESPIRATORY: CTAB, no wheezing, even, unlabored

MSK: left lateral chest wall tenderness, no crepitus, no bruising, no gross deformities of extremities

SKIN: normal color, no diaphoresis

NEURO: steady gait, A&Ox4, no gross deficits

PSYCH: appropriate mood/affect

EKG

None

LABORATORY RESULTS

None

RADIOLOGY

[table#5]:

XR Chest 2 Vw
|---|

Final Result

IMPRESSION:

||

|1. No acute chest abnormality.|

||

||

||

Electronically signed by Ashley Lotfipour 6/9/2023 7:24 PM

||

**TREATMENT AND ED COURSE **

Pertinent Labs & Imaging studies reviewed.

[table#6]:

[Medications]

[---]

[**HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet (1 tablet Oral Given 6/9/23 1903)**]

****Re-evaluation****

The patient was re-evaluated at 7:53 PM PDT. The patient is in no acute distress. CXR results discussed with patient as well as plan for discharge and follow up with SOS clinic for a cardiothoracic surgery consult and to follow up with a dentist for his dental pain. Patient is agreeable to plan and all questions were answered.

****MEDICAL DECISION MAKING****

[table#7]:

[**Medical Decision Making** as of 06/09/23 2315]

[---]

[**Fri Jun 09, 2023**]

[1904 | CURES 6/9/23 at 19:04:

Percocet 5mg #12 filled 3/29/23]

[1905 | Fall off E-bike at the end of March, seen here and diagnosed with 4 rib fractures and trace pleural effusion. Eloped from ED then returned 4 days later with CXR and thoracics consult and advised to f/u with cardiothoracic surgery outpatient. He states he rested for 3 weeks and felt ok on motrin. Over the past 2 weeks has been doing more lifting of boxes and feels his left rib pain is aggravated, no repeat trauma. Also complains of left dental pain that he thinks was from initial e-bike fall]

[1936 | CXR NAD]

[2004 | Patient initially stating he never filled Rx from last visit however CURES states it was filled, patient then states he actually took the drug back to the pharmacy because it wasn't norco and concerned it wasn't the same medication he got in the ED. Concern for pain seeking behavior. He can f/u with SOS clinic to establish primary care for referral to thoracic surgery and needs follow up with a dentist]

** **

This chart was sent for co-signature: NO

****CLINICAL IMPRESSION****

[table#8]:

[**1.**] [**Closed fracture of multiple ribs of left side with routine healing, subsequent encounter **]

[---] [---]

[2.] [Chronic dental pain]

****DISPOSITION****

6/9/2023 7:57 PM PDT

Discussed the historical points, exam findings, and any diagnostic results supporting the presumptive diagnosis. Patient is clinically stable, in no apparent danger of imminent deterioration, and deemed safe for outpatient management. Strict return precautions discussed. Advised to return immediately to the ED if symptoms worsen or persist, or if any concerns arise. All questions answered. Need for appropriate follow up discussed and understood.

****Disposition: DISCHARGED****

Condition: Stable

Discharged to: Home

Follow-up/Referrals:

[table#9]:

[table#9.1]

[table#9.2]

[table#9.3]

[---] [---]

[table#9.1]:

[Follow-up Information]

[---]

[table#9.2]:

[table#9.2.1] Specialty: Public Health

Contact information:

1550 Superior Ave

Costa Mesa California 92627-3653

949-270-2100

[table#9.2.2]

[---] [---]

[table#9.2.1]:

Schedule an appointment as soon as possible for a visit with SOS COMMUNITY HEALTH CENTER.

[table#9.2.2]:

||

[table#9.3]:

||

[table#10]:

||

Prescriptions:

[table#11]:

Discharge Medication List as of 6/9/2023 8:00 PM

||

[table#12]:

START taking these medications

|| Details

acetaminophen (TYLENOL) 500 mg tablet | Take 1-2 tablets by mouth every 6 hours as needed for Pain.Disp-20 tablet, R-0, Normal

||

||

**ATTESTATION **

SCRIBE ATTESTATION

Information entered by Faith Berger acting as scribe for Kerstin Claunch, PA-C

PROVIDER ATTESTATION

The above documentation recorded by the scribe accurately reflects the services I performed and the decisions made by me.

Please note that Dragon Dictation was used to dictate some of this report, and a medical scribe entered much of the historical and clinical information. There is the potential for phonetic inconsistency in how it is translated into text, or information error entry; it is also possible that I might not have found all transcription errors in my review of the record.

Kerstin Jacqueline Claunch, PA-C

06/09/23 2315

Electronically signed by K Claunch, PA at 06/09/2023 11:15 PM PDT

Daniel M - 06/09/2023 6:58 PM PDT

Formatting of this note might be different from the original.

Pt injured left rib area after E bike accident. States pain is worse after moving furniture. Need refill for pain meds.

Electronically signed by Hmhp I at 07/27/2023 11:44 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
<u>Lipid (cholesterol) test</u>	05/04/1983		
<u>COVID-19 Vaccine (#1)</u>	11/04/1983		
<u>DTaP/Tdap/Td Vaccines (1 - Tdap)</u>	05/04/2002		
<u>Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)</u>	05/04/2002		
<u>Pneumococcal Vaccination (1 of 2 - PCV)</u>	05/04/2002		
<u>Influenza Vaccine (#1)</u>	09/01/2026		
HIB Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
HPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Hepatitis A Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
IPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningococcal B Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

XR CHEST 2 VW - Final result (06/09/2023 7:18 PM PDT)

Anatomical Region	Laterality	Modality		
<u>Chest</u>		<u>Radiographic Imaging</u>		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time

Impressions

06/09/2023 7:24 PM PDT

1. No acute chest abnormality.

Electronically signed by Ashley Lotfipour 6/9/2023 7:24 PM

Narrative

06/09/2023 7:24 PM PDT

CHEST, PA AND LATERAL

CLINICAL HISTORY: RIB PAIN
MEDICATION REFILL.Pt injured left rib area after E bike accident. States pain is worse after moving furniture. Need refill for pain meds.

COMPARISON: 3/28/2023

FINDINGS:

The cardiomediastinum, pulmonary parenchyma, pleura, and osseous thorax demonstrate no acute abnormality. Old fracture deformity of the left ribs again seen.

Procedure Note

Dr. A Lotfipour, MD - 07/06/2023

Formatting of this note might be different from the original.

CHEST, PA AND LATERAL

CLINICAL HISTORY: RIB PAIN
MEDICATION REFILL.Pt injured left rib area after E bike accident. States pain is worse after moving furniture. Need refill for pain meds.

COMPARISON: 3/28/2023

FINDINGS:

The cardiomediastinum, pulmonary parenchyma, pleura, and osseous thorax demonstrate no acute abnormality. Old fracture deformity of the left ribs again seen.

IMPRESSION:

1. No acute chest abnormality.

Electronically signed by Ashley Lotfipour 6/9/2023 7:24 PM

Authorizing Provider	Result Type	Result Status
K Claunch PA	IMG XR PROCEDURES	Final Result

Visit Diagnoses - documented in this encounter

Diagnosis

Multiple fractures of ribs, left side, subsequent encounter for fracture with routine healing

Disorder of teeth and supporting structures, unspecified

Insurance - documented as of this encounter

CALOPTIMA MONARCH MG

Member	Subscriber	Plan / Payer (Effective 11/16/2021-Present)
Name: <u>Dimarcello, Anthony</u>	Name: <u>Dimarcello, Anthony</u>	Payer ID: <u>CALOP</u>
Member ID: <u>99451018G</u>	Subscriber ID: <u>99451018G</u>	Group ID: <u>Not on file</u>
Relation to Subscriber: <u>Self</u>		Type: <u>Managed Care Medi-Cal</u>
		Address:
		<u>9 TECHNOLOGY DRIVE</u>
		<u>Irvine, CA 92618</u>

Document Information

Service Providers	Document Coverage Dates
	Jun. 09, 2023
Custodian Organization	
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658	
Encounter Providers	Encounter Date
Dr. E Alcouloumre, MD (Admitting) NPI: 1568425312 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658 Emergency Medicine	Jun. 09, 2023
Legal Authenticator	
Director H	

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
03/28/2023 5:25 PM PDT -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER	Dr. A Cunningham, MD <u>1 Hoag Dr</u> <u>Newport Beach, CA 92658</u> <u>949-764-8372 (Work)</u> <u>949-764-8378 (Fax)</u>	<u>Discharge Disposition: Home or</u> <u>Self Care</u>
03/28/2023 11:19 PM PDT		1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372		

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

No known medications

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Some Days				
Housing Stability		Answer	Date Recorded	
Housing Stability		Not on file	08/05/2023	
Housing Stability		Not on file	08/05/2023	
Interpersonal Safety		Answer	Date Recorded	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Male	01/22/2025 10:41 AM PST	
Legal Sex		Male	05/31/2023 8:08 PM PDT	
Gender Identity		Male	01/22/2025 10:41 AM PST	
Sexual Orientation		Straight	01/22/2025 10:41 AM PST	

Last Filed Vital Signs - documented in this encounter

Not on file

Discharge Instructions - documented in this encounter

Discharge Instructions
B Felter, NP - 03/28/2023 7:58 PM PDT
Formatting of this note is different from the original.
Discharge Instructions by Brenda Marie Felter, FNP at 03/28/23 1958
Author: Brenda Marie Felter, FNP Service: -- Author Type: Nurse Practitioner
Filed: 03/28/23 2003 Date of Service: 03/28/23 1958 Creation Time: 03/28/23 1958
Status: Edited Editor: Brenda Marie Felter, FNP (Nurse Practitioner)
Related Notes: Original Note by Brenda Marie Felter, FNP (Nurse Practitioner) filed at 03/28/23 1959

Your injury and imaging has been reviewed by cardiothoracic surgery. They have cleared you for outpatient follow-up. A referral to their clinic is provided below. Please call in the a.m. to establish an appointment.

Electronically signed by B Felter, NP at 03/28/2023 8:03 PM PDT

ED Notes - documented in this encounter

B Felter, NP - 03/28/2023 6:22 PM PDT
Formatting of this note might be different from the original.

[table#1]:
ED Provider Notes by Brenda Marie Felter, FNP at 03/28/23 1822

[table#2]:
ED Provider Notes by Brenda Marie Felter, FNP at 03/28/23 1822 (continued)
[image]
EMERGENCY DEPARTMENT HISTORY AND PHYSICAL
HOAG HOSPITAL PRESBYTERIAN

3/28/2023
6:22 PM PDT

Patient Name: Anthony Dimarcello **DOB**: 5/4/1983 **Medical Record**: 20012489767 ** **
Room: LDC
PCP: No Physician on file

My Supervising physician is Dr. Cunningham

****CHIEF COMPLAINT****

[table#3]:

****Chief Complaint**** |

| --- |

| Patient presents with |

| • | Shortness of Breath |

| • | Chest Pain |

| • | Cough |

****HPI****

Anthony Dimarcello is a 39 y.o. male who presents with chest pain onset 3 days ago s/p falling off electric bike 3 days ago for which he was seen at the ED, however later eloped. Patient reports 10/10 chest pain accompanied with SOB and cough. Patient also notes a healing laceration to RLE. Patient reports he's "filling up with fluids." Per chart review on 3/24/23, patient was seen here at the ED for evaluation of left shoulder pain, left chest pain, and RLE pain s/p being "slingshot" from his heavy E-bike. Chest CT shows 4 rib fractures which 2 were mildly displaced. Recommended outpatient follow-up and pain medication however patient eloped before he could be reevaluated.

****History Obtained from Independent historian****: None

****PAST MEDICAL/SURGICAL HISTORY****

History reviewed. No pertinent past medical history.

History reviewed. No pertinent surgical history.

****CURRENT MEDICATIONS****

[table#4]:

| No current outpatient medications on file prior to encounter. |

| --- |

****PHYSICAL EXAM****

****Vital Signs****: Temp: 36.9 °C (98.5 °F) Pulse: 113 Resp: 18 BP: 107/64 SpO2: 97 %

Gen- WDOWN, Alert and Oriented, mild distress

HEENT- NC/NT, PERRLA, EOMI, Hearing grossly intact.

Neck- Supple, no midline tenderness, FROM without pain

CVA-tachycardic, no murmur, left-sided chest wall pain

Resp- lungs equal and clear to auscultation B, no resp. Distress

Abd- Soft, NTTP, NABS, no guarding, no rebound tenderness

GU- no CVA tenderness

Back- NTTP, FROM, no midline tenderness

Musc.- Extremities Nontender, no edema, no obvious deformities.

Neuro/Psych- AxO to 4, CN 2-10 grossly intact, motor normal, sensation normal, mood/affect normal.

-

****LABORATORY RESULTS****

[table#5]:

****Recent Results (from the past 24 hour(s))**** |

| --- |

****ECG 12 lead**** |

| Result | Value | Ref Range |

|| INTERPRETATION TEXT | Not confirmed ||

****RADIOLOGY****

[table#6]:

| XR Chest 2 Vw |

| --- |

****Final Result**** |

| IMPRESSION: |

||

| 1. Trace left effusion. |

2. Known left-sided rib fractures which are better seen on the recent CT |

Electronically signed by Scott Williams 3/28/2023 6:50 PM |

MEDICAL DECISION MAKING

Review of External Records: ED visit note on 3/24/23 reviewed. Per chart review on 3/24/23, patient was seen here at the ED for evaluation of left shoulder pain, left chest pain, and RLE pain s/p being "slingshot" from his heavy E-bike. Chest CT shows 4 rib fractures which 2 were mildly displaced. Recommended outpatient follow-up and pain medication however patient eloped before he could be reevaluated.

EKG Interpretation:

EKG reviewed at 7:31 PM PDT and independently interpreted by me.

Rate: 117

Rhythm: sinus tachycardia

Axis: normal

ST segments: normal.

Conduction and Intervals: normal

No prior EKG available for comparison

Labs/Imaging:

All laboratory tests were independently reviewed and interpreted by myself.

N/A

Treatments Provided

[table#7]:

Medications |

HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet (1 tablet Oral Given 3/28/23 1806) |

ibuprofen (ADVIL, MOTRIN) tablet 600 mg (600 mg Oral Given 3/28/23 1805) |

[table#8]:

Orders Placed This Encounter |

Procedures |

• | XR Chest 2 Vw |

• | New VS plz |

• | ECG 12 lead |

CONSULT

Dr. Nguyen Le, Thoracic Surgeon, consulted at 7:54 PM PDT. Discussed patient's case. Agrees with Evaluation and Plan. Recommends outpatient f/u.

Re-evaluation

The patient was re-evaluated at 7:59 PM PDT.

Patient does not appear to be in any acute distress. Updated on Chest X-Ray results. Discussed plan of care and answered all available questions. Patient is agreeable and understands.

Discussion/Plan:

Patient returns the emergency department with reported difficulty breathing and sensation of fluid in lungs. I saw this patient during his last visit s/p E bike injury with subsequent rib fractures. At that time there was no evidence of pulmonary contusion, pneumothorax, hemothorax, or pleural effusion. Cardiothoracic was consulted during that visit who recommended outpatient follow-up but patient had eloped prior to formal discharge discussion. Today he is afebrile with mild tachycardia and moderate pain. Lungs are still clear on exam. Chest x-ray shows trace pleural effusion. Cardiothoracic's reconsulted who still recommends outpatient follow-up. Patient given an incentive spirometer with education as well as appropriate pain medication. Discussed suggested outpatient follow-up as well as precautions.

BP 125/87 | Pulse 87 | Temp 36.9 °C (98.4 °F) (Oral) | Resp 16 | Ht 1.803 m (5' 11") | Wt 91.2 kg (201 lb 1 oz) | SpO2 100% | BMI 28.04 kg/m²

Risk of Complications/Morbidity/Mortality of patient Management:

This patient encounter included: Decision regarding hospitalization

CLINICAL IMPRESSION

[table#9]:

**1. ** Closed fracture of multiple ribs of left side with routine healing, subsequent encounter ** |

[---] [---]
[2.] Pleural effusion on left
[3.] Contusion of right lower extremity, initial encounter

This list includes current acute medical illnesses as well as chronic conditions addressed during this visit.

DISPOSITION
3/28/2023 8:12 PM PDT

Discussed the historical points, exam findings, and any diagnostic results supporting the presumptive diagnosis. Patient is clinically stable, in no apparent danger of imminent deterioration, and deemed safe for outpatient management. Strict return precautions discussed. Advised to return immediately to the ED if symptoms worsen or persist, or if any concerns arise. All questions answered. Need for appropriate follow up discussed and understood.

Disposition: DISCHARGED

Condition: Stable
Discharged to: Home
Follow-up/Referrals:

[table#10]:
[table#10.1]
[table#10.2]
[table#10.3]
[---] [---]

[table#10.1]:
Follow-up Information
[---]

[table#10.2]:
[table#10.2.1] Specialty: Thoracic Surgery
Contact information:
1 HOAG DR
BLDG 39
Newport Beach CA 92663
949-764-6166

[table#10.2.2]:
[---] [---]

[table#10.2.1]:
Nguyen M. Le, MD. Schedule an appointment as soon as possible for a visit in 1 week.
[---]

[table#10.2.2]:
[---]

[table#10.3]:
[---]

[table#11]:
[---]

Prescriptions:
[table#12]:
Discharge Medication List as of 3/28/2023 11:09 PM
[---]
[---]

[table#13]:
START taking these medications
[---]
[---] Details
HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet | Take 1 tablet by mouth every 6 hours as needed for Pain for up to 3 days.Disp-12 tablet, R-0, Normal
[---]
ibuprofen (ADVIL,MOTRIN) 600 MG tablet | Take 1 tablet by mouth every 6 hours as needed for Pain.Disp-30 tablet, R-0, Normal
[---]

||

****ATTESTATION ****

****SCRIBE ATTESTATION****

Information entered by Harrison Nguyen acting as scribe for Brenda Felter, FNP.

****MD ATTESTATION****

The above documentation recorded by the scribe accurately reflects the services I performed and the decisions made by me.

Please note that Dragon Dictation was used to dictate some of this report, and a medical scribe entered much of the historical and clinical information. There is the potential for phonetic inconsistency in how it is translated into text, or information error entry; it is also possible that I might not have found all transcription errors in my review of the record.

Brenda Marie Felter, FNP
03/29/23 0232

[table#14]:

||

[---]

Electronically signed by B Felter, NP at 03/29/2023 2:32 AM PDT

Nurse Hilary P - 03/28/2023 5:37 PM PDT

Formatting of this note might be different from the original.

CC fall off electric bike 3 days ago. Was seen in Hoag ED. Has healing lac to RLE. +cough +SOB. +CP 10/10. Reports he's "filling up with fluids"

Electronically signed by Nurse Hilary P at 03/28/2023 5:38 PM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Lipid (cholesterol) test	05/04/1983		
COVID-19 Vaccine (#1)	11/04/1983		
DTaP/Tdap/Td Vaccines (1 - Tdap)	05/04/2002		
Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)	05/04/2002		
Pneumococcal Vaccination (1 of 2 - PCV)	05/04/2002		
Influenza Vaccine (#1)	09/01/2026		
HIB Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
HPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Hepatitis A Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
IPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningococcal B Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

12 Lead Electrocardiogram (ECG 12 LEAD) - Final result (03/28/2023 11:59 PM PDT)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
<u>VENTRICULAR RATE EKG</u>	<u>117</u>	<u>bpm</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>ATRIAL RATE</u>	<u>116</u>	<u>bpm</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>P-R INTERVAL</u>	<u>149</u>	<u>ms</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>P WAVE AXIS</u>	<u>71</u>	<u>deg</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>QRS DURATION</u>	<u>74</u>	<u>ms</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>Q-T INTERVAL</u>	<u>340</u>	<u>ms</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>Q-T INTERVAL (CORRECTED)</u>	<u>473</u>	<u>ms</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>QRS AXIS</u>	<u>75</u>	<u>deg</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>T AXIS</u>		<u>deg</u>		<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	
<u>INTERPRETATION TEXT</u>	<u>- BORDERLINE ECG - Sinus tachycardia Borderline repolarization abnormality No previous ECG available for comparison Electronically signed by: Parto, Parham 29-Mar-2023 15:33:01</u>			<u>03/29/2023 3:33 PM PDT</u>	<u>CA HH IECG</u>	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			<u>03/28/2023 5:35 PM PDT</u>	

Narrative

This result has an attachment that is not available.

-

Authorizing Provider	Result Type	Result Status
<u>B Felter NP</u>	<u>ECG ORDERABLES</u>	<u>Final Result</u>

Performing Organization	Address	City/State/ZIP Code	Phone Number
<u>HMHP CONVERSION IMAGING</u>			-
<u>CA HH IECG</u>			-

XR CHEST 2 VW - Final result (03/28/2023 6:30 PM PDT)

Anatomical Region	Laterality	Modality		
Chest		Radiographic Imaging		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Impressions				
03/28/2023 6:50 PM PDT				
1. Trace left effusion.				
2. Known left-sided rib fractures which are better seen on the recent CT				
Electronically signed by Scott Williams 3/28/2023 6:50 PM				
Narrative				
03/28/2023 6:50 PM PDT				
CHEST, PA AND LATERAL				
CLINICAL HISTORY: Lt rib fx. worsening pain.				
COMPARISON: CT 3/24/2023				
FINDINGS:				
Left-sided rib fractures are better seen on the recent CT. There is some trace fluid at the left base. No pneumothorax. Heart size is normal.				
—				
Procedure Note				
Dr. S Williams, MD - 07/01/2023				
Formatting of this note might be different from the original.				
CHEST, PA AND LATERAL				
CLINICAL HISTORY: Lt rib fx. worsening pain.				
COMPARISON: CT 3/24/2023				
FINDINGS:				
Left-sided rib fractures are better seen on the recent CT. There is some trace fluid at the left base. No pneumothorax. Heart size is normal.				
IMPRESSION:				
1. Trace left effusion.				
2. Known left-sided rib fractures which are better seen on the recent CT				
Electronically signed by Scott Williams 3/28/2023 6:50 PM				
Authorizing Provider	Result Type	Result Status		
B Felter NP	IMG XR PROCEDURES	Final Result		

Visit Diagnoses - documented in this encounter

Diagnosis

Multiple fractures of ribs, left side, subsequent encounter for fracture with routine healing
Pleural effusion, not elsewhere classified
Contusion of right lower leg, initial encounter

Insurance - documented as of this encounter

CALOPTIMA MONARCH MG

Member	Subscriber	Plan / Payer (Effective 11/16/2021-Present)
Name: <u>Dimarcello, Anthony</u> Member ID: <u>99451018G</u> Relation to Subscriber: <u>Self</u>	Name: <u>Dimarcello, Anthony</u> Subscriber ID: <u>99451018G</u>	Payer ID: <u>CALOP</u> Group ID: <u>Not on file</u> Type: <u>Managed Care Medi-Cal</u> Address: <u>9 TECHNOLOGY DRIVE</u> <u>Irvine, CA 92618</u>

Document Information

Service Providers	Document Coverage Dates
	Mar. 28, 2023
Custodian Organization	
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658	
Encounter Providers	Encounter Date
Dr. A Cunningham, MD (Admitting) NPI: 1952795999 949-764-8372 (Work) 949-764-8378 (Fax) 1 Hoag Dr Newport Beach, CA 92658 Emergency Medicine	Mar. 28, 2023
Legal Authenticator	
Director H	

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
03/24/2023 12:20 AM PDT -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER	Dr. D Ritter, MD 1 Hoag Dr Newport Beach, CA 92658 949-764-8372 (Work) 949-764-8378 (Fax)	Discharge Disposition: LEFT AGAINST MEDICAL ADVICE OR DISCONTINUED CARE
03/24/2023 3:20 AM PDT		1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372		

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

No known medications

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Some Days				
Housing Stability	Answer		Date Recorded	
Housing Stability	Not on file		08/05/2023	
Housing Stability	Not on file		08/05/2023	
Interpersonal Safety	Answer		Date Recorded	
Interpersonal Safety	0		08/05/2023	
Interpersonal Safety	0		08/05/2023	
Interpersonal Safety	0		08/05/2023	
Interpersonal Safety	0		08/05/2023	
Sex and Gender Information	Value		Date Recorded	
Sex Assigned at Birth	Male		01/22/2025 10:41 AM PST	
Legal Sex	Male		05/31/2023 8:08 PM PDT	
Gender Identity	Male		01/22/2025 10:41 AM PST	
Sexual Orientation	Straight		01/22/2025 10:41 AM PST	

Last Filed Vital Signs - documented in this encounter

Not on file

Discharge Instructions - documented in this encounter

Discharge Instructions
B Felter, NP - 03/24/2023 3:20 AM PDT
Formatting of this note is different from the original.
Discharge Instructions by Brenda Marie Felter, FNP at 03/24/23 0333
Author: Brenda Marie Felter, FNP Service: -- Author Type: Nurse Practitioner
Filed: 03/24/23 0333 Date of Service: 03/24/23 0333 Creation Time: 03/24/23 0333
Status: Written Editor: Brenda Marie Felter, FNP (Nurse Practitioner)

Electronically signed by B Felter, NP at 03/24/2023 3:33 AM PDT

ED Notes - documented in this encounter

B Felter, NP - 03/24/2023 12:56 AM PDT
Formatting of this note might be different from the original.

[table#1]:
ED Provider Notes by Brenda Marie Felter, FNP at 03/24/23 0056

[table#2]:
ED Provider Notes by Brenda Marie Felter, FNP at 03/24/23 0056 (continued)
[image]
EMERGENCY DEPARTMENT HISTORY AND PHYSICAL
HOAG HOSPITAL PRESBYTERIAN

3/24/2023
12:56 AM PDT

Patient Name: Anthony Dimarcello **DOB**: 5/4/1983 **Medical Record**: 20012489767 ** **
Room: LDC
PCP: No Physician on file

My Supervising physician is Dr. Daniel Ritter

CHIEF COMPLAINT
[table#3]:

****Chief Complaint****

Patient presents with

- Fall
- Chest Pain
- Shortness of Breath

****HPI****

Anthony Dimarcello is a 39 y.o. male who presents for evaluation of left shoulder pain, left chest pain, and right lower leg pain after being "slingshot" from his heavy E-bike about 24 hours ago. He states he was riding his E-bike along a dark path toward a large hill, pursued by 4 strangers on regular bicycles. He outrode his pursuers and attempted to turn to avoid striking the nearing hill but was moving too quickly. He attempted to slide, but his bike was too heavy and he was "slingshot" to the other side, landing on his left side on concrete. States his right lower leg was also wounded from his bike's spiked pedals but denies head trauma or LOC. States he smoked some marijuana following the accident with minimal relief. He currently reports intermittent dull left-sided chest pain and sharp, constant right lower leg pain with radiation to his ankle and up his shin. He also reports feeling "something swirling around" in his left lateral ribs beneath his left arm and states he has felt something in his left shoulder "rising up" throughout the day yesterday. States he felt a "pop" in his left upper back. He has been stretching his left shoulder but notes worsened lower left chest pain when moving his left arm. States his current pain is the worst he has ever experienced.

****PAST MEDICAL/SURGICAL HISTORY****

Patient denies any relevant past medical or surgical history.

****CURRENT MEDICATIONS****

[table#4]:

No current outpatient medications on file prior to encounter.

****PHYSICAL EXAM****

****Vital Signs:**** Temp: 36.6 °C (97.9 °F) Pulse: 107 Resp: 18 BP: 124/88 SpO2: 98 %

General: well-appearing, well nourished, non-toxic appearing

Head: normocephalic, atraumatic

Ears: hearing intact

Eyes: non-icteric, sclera white, no discharge

Nose: no rhinorrhea or epistaxis, no nasal flaring

Cardiovascular: good peripheral perfusion

Chest: No chest wall deformity or overlying abrasion/ecchymosis. Tenderness on palpation of left anterior lateral ribs without underlying crepitus.

Lungs: no respiratory distress, speaks full sentences without difficulty

Skin: no obvious rashes

Musc: LUE: Normal joint appearance without deformity or edema. Normal ROM. Reported pain with extremity abduction and extension. Distal extremity neurovascularly intact with normal pulses and cap refill. Sensation intact. RLE: Palpable hematoma to the distal medial calf with small overlying abrasion. Normal pulses and cap refill distal to injury.

Neuro: alert and oriented, GCS 15

Psych: normal mood and affect

****LABORATORY RESULTS****

N/A

****RADIOLOGY****

[table#5]:

CT Chest Abdomen Pelvis wo Contrast

XR Tibia Fibula Right 2 Vw (Results Pending)

XR Ankle Right 3 + Vw (Results Pending)

XR Shoulder Left 2 + Vw (Results Pending)

Right Tibia Fibula XR: No acute findings.

Right Ankle XR: No acute findings.

Left Shoulder XR: No acute findings.

CT Chest Abdomen Pelvis wo Contrast:

PRELIMINARY REPOR

I

1. Mildly displaced fractures of the left lateral 5th and 6th ribs and nondisplaced fractures of the left anterolateral 3rd and 4th ribs. No pneumothorax.

2. 1.2 cm peribronchial density in the left upper lobe (series 3 image 84), possibly infectious/inflammatory, less likely contusion. 1.8 cm peri-fissural density in the medial right lower lobe (series 3 image 107). Recommend follow-up CT chest.

This is a preliminary report provided for triage and management of acute findings. Please refer to the detailed, final report when available.

Read by Brian Dubin, MD at 02:11 PDT

****MEDICAL DECISION MAKING****

****Review of External Records:**** External ED visit on 12/17/22 at Long Beach Memorial ED for left jaw pain and right foot pain sustained while working as an Uber driver.

****EKG Interpretation:****

No EKG performed.

****Imaging:****

Right Tibia Fibula, Right Ankle, and Left Shoulder X-ray independently reviewed and interpreted by myself which show no acute findings.

****Treatments Provided****

[table#6]:

| Medications |

| --- |

| ****HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet (1 tablet Oral Given 3/24/23 0134)**** |

| ****ibuprofen (ADVIL, MOTRIN) tablet 600 mg (600 mg Oral Given 3/24/23 0134)**** |

[table#7]:

| ****Orders Placed This Encounter**** |

| --- |

| Procedures |

| • | XR Tibia Fibula Right 2 Vw |

| • | XR Ankle Right 3 + Vw |

| • | XR Shoulder Left 2 + Vw |

| • | CT Chest Abdomen Pelvis wo Contrast |

| • | Phone Consult - HUC Communication |

****Consults****

Dr. Brian Dubin (Radiology) consulted at 2:37 AM PDT.

States CT Chest wo Contrast can be done out-patient.

Dr. Timothy Lee (Cardiovascular Surgery), on call for Dr. Daryl Pearlstein, consulted at 3:00 AM PDT.

Case reviewed. Recommends out-patient follow-up at Dr. Pearlstein's office and pain medications.

****Re-evaluations****

Called patient's name at 3:03 AM PDT in the lobby with no response.

Called patient's name again at 3:18 AM PDT in the lobby with no response.

Called patient's name at 3:19 AM PDT in the tent outside with no response.

****Discussion/Plan:****

[table#8]:

| ****Medical Decision Making**** as of 03/24/23 0420 |

| --- |

| ****Fri Mar 24, 2023**** |

| 0257 | Patient presents with left chest wall pain, left shoulder pain, and right lower extremity pain s/p fall off of an E bike 24 hours ago. No head injury or loss of consciousness. Patient attempted self-medication with marijuana with minimal relief. Chest wall pain exacerbated by inspiration and movement. Patient is hemodynamically stable with an SPO2 of 98% on room air. No significant overlying abrasions or ecchymosis but definite pain on palpation of ribs. No palpable crepitus. Patient has left shoulder pain without obvious deformity or edema. X-ray unremarkable for fracture or dislocation. Right lower extremity has a hematoma and abrasion. No evidence of foreign body, fracture, or dislocation per x-ray. Chest CT shows 4 rib fractures which 2 were mildly displaced. |

| 0319 | Cardiothoracic's consulted and case reviewed. Recommends outpatient follow-up and pain medication. Attempted to reevaluate patient and discussed disposition but patient eloped from ED. |

****Risk of Complications/Morbidity/Mortality of patient Management: ****

This patient encounter included: Decision regarding elective/emergency procedure and Decision regarding hospitalization

****CLINICAL IMPRESSION****

[table#9]:

1. | **Closed fracture of multiple ribs of left side, initial encounter ** |

| --- | --- |

| 2. | Hematoma of right lower extremity, initial encounter |

This list includes current acute medical illnesses as well as chronic conditions addressed during this visit.

DISPOSITION

3/24/2023 3:20 AM PDT

The patient eloped from the Emergency Department prior to completion of evaluation. They could not be found in their assigned ED location or in surrounding areas.

Disposition: Eloped

**ATTESTATION **

SCRIBE ATTESTATION

Information entered by Nikki Young acting as scribe for Brenda Felter, FNP.

MD ATTESTATION

The above documentation recorded by the scribe accurately reflects the services I performed and the decisions made by me.

Please note that Dragon Dictation was used to dictate some of this report, and a medical scribe entered much of the historical and clinical information. There is the potential for phonetic inconsistency in how it is translated into text, or information error entry; it is also possible that I might not have found all transcription errors in my review of the record.

Brenda Marie Felter, FNP

03/24/23 0421

[table#10]:

||

| --- |

Electronically signed by B Felter, NP at 03/24/2023 4:21 AM PDT

Nurse Sakol C - 03/24/2023 12:34 AM PDT

Formatting of this note might be different from the original.

Pt reporting the he fell off his bike and is complaining of L CP and R leg pain.

Electronically signed by Nurse Sakol C at 03/24/2023 12:35 AM PDT

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
<u>Lipid (cholesterol) test</u>	05/04/1983		
<u>COVID-19 Vaccine (#1)</u>	11/04/1983		
<u>DTaP/Tdap/Td Vaccines (1 - Tdap)</u>	05/04/2002		
<u>Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)</u>	05/04/2002		
<u>Pneumococcal Vaccination (1 of 2 - PCV)</u>	05/04/2002		
<u>Influenza Vaccine (#1)</u>	09/01/2026		
<u>HIB Vaccine</u>	Aged Out		No longer eligible based on patient's age to complete this topic
<u>HPV Vaccine</u>	Aged Out		No longer eligible based on patient's age to complete this topic
<u>Hepatitis A Vaccine</u>	Aged Out		No longer eligible based on patient's age to complete this topic
<u>IPV Vaccine</u>	Aged Out		No longer eligible based on patient's age to complete this topic
<u>Meningitis Vaccine</u>	Aged Out		No longer eligible based on patient's age to complete this topic
<u>Meningococcal B Vaccine</u>	Aged Out		No longer eligible based on patient's age to complete this topic
<u>Rotavirus Vaccine</u>	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

Computed Tomography (CT) Chest Abdomen Pelvis Without Contrast (CT CHEST ABDOMEN PELVIS WO CONTRAST) - Final result (03/24/2023 1:51 AM PDT)

Anatomical Region		Laterality	Modality		
<u>Body, Chest</u>			<u>Computed Tomography</u>		
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume		Collection Time
					Received Time

Impressions

03/24/2023 8:06 AM PDT

1. Non to mildly displaced fractures of the left 3rd through 6th ribs without evidence for pneumothorax.

2. 1.2 cm peribronchial density in the left upper lobe probably infectious/inflammatory given associated bronchiectasis. 1.8 cm peri-fissural density in the medial right lower lobe. Recommend follow-up CT chest.

Electronically signed by Peter Giuliano 3/24/2023 8:06 AM

Narrative

03/24/2023 8:06 AM PDT

CT CHEST, ABDOMEN, AND PELVIS WITHOUT CONTRAST

CLINICAL HISTORY: Fall. Left-sided chest and abdominal pain

COMPARISON: None.

TECHNIQUE: Using a multidetector helical CT scanner, thin slice acquisition was performed through the chest, abdomen, and pelvis without intravenous contrast. Multiplanar reformations were obtained. CT scan done according to ALARA.

DOSE INFORMATION:

CTDIvol (mGy)

Series 2 - Spiral - 12.84 (mGy)

Total DLP (mGy-cm): 885.50 mGy.cm.

(Note: The above reported CTDIvol and DLP values are CT scanner radiation output related dose indices, and, as such, they do NOT represent actual patient dose estimates. A medical physicist should be consulted for specific questions regarding the radiation dose for this exam).

Note that evaluation of the solid organs, soft tissues, and vascular structures is limited without intravenous contrast.

CHEST FINDINGS:

Airways: Unremarkable.

Lungs: 1.8 cm x 0.5 cm infiltrate in the right lower lobe posterior to the heart on image 107 series 3. Small nodular infiltrate measuring just over centimeter in the left upper lobe on image 88 series 3 with adjacent bronchiectasis

Pleura: No effusion or thickening.

Heart and Pericardium: Heart size is normal. No pericardial effusion.

Coronary Artery Calcification: Absent.

Vessels: Unremarkable.

Mediastinum and Hila: Unremarkable.

Chest wall and Lower Neck: Unremarkable.

ABDOMEN AND PELVIS FINDINGS:

Liver: Unremarkable.

Gallbladder and Biliary Tree: Unremarkable.

Pancreas: Unremarkable.

Spleen: Unremarkable.

Adrenals: Unremarkable.

Kidneys and Ureters: Unremarkable.

Bladder: Unremarkable.

Reproductive Organs: Unremarkable.

Bowel: Unremarkable.

Lymph Nodes: No obvious lymphadenopathy.

Peritoneum: No free air, free fluid, or fluid collections.

Vessels: Unremarkable.

Abdominal wall: Unremarkable.

BONE FINDINGS: Non to minimally displaced fractures of the lateral/anterolateral left 3rd through 6th ribs

—

Dr. P Giuliano, MD - 07/01/2023

Formatting of this note might be different from the original.

CT CHEST, ABDOMEN, AND PELVIS WITHOUT CONTRAST

CLINICAL HISTORY: Fall. Left-sided chest and abdominal pain

COMPARISON: None.

TECHNIQUE: Using a multidetector helical CT scanner, thin slice acquisition was performed through the chest, abdomen, and pelvis without intravenous contrast. Multiplanar reformations were obtained. CT scan done according to ALARA.

DOSE INFORMATION:

CTDIvol (mGy)

Series 2 - Spiral - 12.84 (mGy)

Total DLP (mGy-cm): 885.50 mGy.cm.

(Note: The above reported CTDIvol and DLP values are CT scanner radiation output related dose indices, and, as such, they do NOT represent actual patient dose estimates. A medical physicist should be consulted for specific questions regarding the radiation dose for this exam).

Note that evaluation of the solid organs, soft tissues, and vascular structures is limited without intravenous contrast.

CHEST FINDINGS:

Airways: Unremarkable.

Lungs: 1.8 cm x 0.5 cm infiltrate in the right lower lobe posterior to the heart on image 107 series 3. Small nodular infiltrate measuring just over centimeter in the left upper lobe on image 88 series 3 with adjacent bronchiectasis

Pleura: No effusion or thickening.

Heart and Pericardium: Heart size is normal. No pericardial effusion.

Coronary Artery Calcification: Absent.

Vessels: Unremarkable.

Mediastinum and Hila: Unremarkable.

Chest Wall and Lower Neck: Unremarkable.

ABDOMEN AND PELVIS FINDINGS:

Liver: Unremarkable.

Gallbladder and Biliary Tree: Unremarkable.

Pancreas: Unremarkable.

Spleen: Unremarkable.

Adrenals: Unremarkable.

Kidneys and Ureters: Unremarkable.

Bladder: Unremarkable.

Reproductive Organs: Unremarkable.

Bowel: Unremarkable.

Lymph Nodes: No obvious lymphadenopathy.

Peritoneum: No free air, free fluid, or fluid collections.

Procedure Note

Vessels: Unremarkable.

Abdominal Wall: Unremarkable.

BONE FINDINGS: Non to minimally displaced fractures of the lateral/anterolateral left 3rd through 6th ribs

IMPRESSION:

- 1. Non to mildly displaced fractures of the left 3rd through 6th ribs without evidence for pneumothorax.
- 2. 1.2 cm peribronchial density in the left upper lobe probably infectious/inflammatory given associated bronchiectasis. 1.8 cm peri-fissural density in the medial right lower lobe. Recommend follow-up CT chest.

Electronically signed by Peter Giuliano 3/24/2023 8:06 AM

Authorizing Provider	Result Type	Result Status
B Felter NP	IMG CT PROCEDURES	Final Result

XR SHOULDER LEFT 2 PLUS VW - Final result (03/24/2023 1:27 AM PDT)

Anatomical Region	Laterality	Modality
Upper Extremities, Shoulder	Left	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:44 AM PDT

1. Multiple minimally displaced left-sided rib fractures. No significant pneumothorax No acute osseous abnormality of the left shoulder.

Electronically signed by Peter Giuliano 3/24/2023 7:44 AM

Narrative

03/24/2023 7:44 AM PDT

LEFT SHOULDER, 4 VIEWS

CLINICAL HISTORY: Fall. Pain. COMPARISON: None.

FINDINGS:

Several nondisplaced fractures of the lateral left rib cage noted approximately at the level of the 5th through 7th ribs No other acute fracture or dislocation. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

Procedure Note

Dr. P Giuliano, MD - 07/01/2023

Formatting of this note might be different from the original.

LEFT SHOULDER, 4 VIEWS

CLINICAL HISTORY: Fall. Pain. COMPARISON: None.

FINDINGS:

Several nondisplaced fractures of the lateral left rib cage noted approximately at the level of the 5th through 7th ribs No other acute fracture or dislocation. The glenohumeral joint is unremarkable. The acromioclavicular joint is unremarkable. Visualized lungs are clear. No significant soft tissue swelling.

IMPRESSION:

1. Multiple minimally displaced left-sided rib fractures. No significant pneumothorax No acute osseous abnormality of the left shoulder.

Electronically signed by Peter Giuliano 3/24/2023 7:44 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

X-Ray (XR) Ankle Right 3 or more views (XR ANKLE 3+ VW RIGHT) - Final result (03/24/2023 1:27 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Ankle	Right	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:44 AM PDT

1. No acute osseous abnormality of the right ankle.

Electronically signed by Shaya Ghazinoor 3/24/2023 7:44 AM

Narrative

03/24/2023 7:44 AM PDT

RIGHT ANKLE, 3 VIEWS

CLINICAL HISTORY: Ankle pain status post fall.

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The ankle mortise is symmetric and the other joint spaces are preserved. No significant soft tissue swelling.

Formatting of this note might be different from the original.

RIGHT ANKLE, 3 VIEWS

CLINICAL HISTORY: Ankle pain status post fall.

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The ankle mortise is symmetric and the other joint spaces are preserved. No significant soft tissue swelling.

IMPRESSION:

1. No acute osseous abnormality of the right ankle.

Electronically signed by Shaya Ghazinoor 3/24/2023 7:44 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

X-Ray (XR) Tibia Fibula Right 2 views (XR TIBIA FIBULA 2 VIEWS RIGHT) - Final result (03/24/2023 1:26 AM PDT)

Anatomical Region	Laterality	Modality
Lower Extremities, Lower Leg	Right	Digital Radiography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
-------------------	----------------------------------	----------------------------	-----------------	---------------

Impressions

03/24/2023 7:42 AM PDT

1. No acute osseous abnormality of the right tibia and fibula.

Electronically signed by Peter Giuliano 3/24/2023 7:42 AM

Narrative

03/24/2023 7:42 AM PDT

RIGHT TIBIA AND FIBULA, 2 VIEWS

CLINICAL HISTORY: Pain midshaft; fall off bike with pain

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The joint spaces are preserved. No significant soft tissue swelling. No radiopaque foreign body

Formatting of this note might be different from the original.

RIGHT TIBIA AND FIBULA, 2 VIEWS

CLINICAL HISTORY: Pain midshaft; fall off bike with pain

COMPARISON: None.

FINDINGS:

No acute fracture or malalignment. The joint spaces are preserved. No significant soft tissue swelling. No radiopaque foreign body

IMPRESSION:

1. No acute osseous abnormality of the right tibia and fibula.

Electronically signed by Peter Giuliano 3/24/2023 7:42 AM

Authorizing Provider	Result Type	Result Status
K Bagiu NP	IMG XR PROCEDURES	Final Result

Visit Diagnoses - documented in this encounter

Diagnosis
Multiple fractures of ribs, left side, initial encounter for closed fracture
Contusion of right lower leg, initial encounter

Insurance - documented as of this encounter

CALOPTIMA MONARCH MG

Member	Subscriber	Plan / Payer (Effective 11/16/2021-Present)
Name: Dimarcello, Anthony	Name: Dimarcello, Anthony	Payer ID: CALOP
Member ID: 99451018G	Subscriber ID: 99451018G	Group ID: Not on file
Relation to Subscriber: Self		Type: Managed Care Medi-Cal
		Address:
		9 TECHNOLOGY DRIVE
		Irvine, CA 92618

Document Information

Service Providers	Document Coverage Dates
	Mar. 24, 2023
Custodian Organization	
HOAG MEMORIAL HOSPITAL PRESBYTERIAN	
One Hoag Drive	
Newport Beach, CA 92658	
Encounter Providers	Encounter Date
Dr. D Ritter, MD (Admitting)	Mar. 24, 2023
NPI: 1770004608	
949-764-8372 (Work)	
949-764-8378 (Fax)	
1 Hoag Dr	
Newport Beach, CA 92658	
Emergency Medicine	
Legal Authenticator	
Director H	

Anthony Dimarcello "Anthony"

Summary of Care, generated on Aug. 29, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
Former (Oct. 30, 2023 - Feb. 10, 2024): 412 Olive Ave (Home) Huntington Beach, CA 92648	Anthony Dimarcello	949-350-2312 (Home) 949-350-2312 (Mobile) amd949609@gmail.com AMD949609@GMAIL.COM
(Feb. 24, 2026 -): 412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Former / Aliases: Anthony Dimarcello Scarlet November Octagon Copper Papa Rectangle	
Former (Aug. 29, 2024 - Feb. 23, 2026): 412 Olive Ave Ste 200 (Home) HUNTINGTN BCH, CA 92648		
Former (Feb. 11, 2024 - Aug. 28, 2024): Pt States No Address (Home) NEWPORT BEACH, CA 92663		
Language	Race / Ethnicity	Marital Status
English - Spoken (Preferred) English - Written (Preferred)	White / Not Hispanic or Latino	Unknown

Note from HOAG MEMORIAL HOSPITAL PRESBYTERIAN

This document contains information that was shared with Anthony Dimarcello. It may not contain the entire record from HOAG MEMORIAL HOSPITAL PRESBYTERIAN.

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
08/20/2014 1:37 PM PDT -	Emergency	HOAG MEMORIAL HOSPITAL NEWPORT EMERGENCY CENTER 1 HOAG DR NEWPORT BEACH, CA 92663-4162 949-764-8372	PROVIDER, HOAG IP CONVERSION	<u>Discharge Disposition: Home or Self Care</u>

Allergies - documented as of this encounter (statuses as of 08/29/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/29/2026)

No known medications

Active Problems - documented as of this encounter (statuses as of 08/29/2026)

Not on file

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Some Days				
Housing Stability		Answer	Date Recorded	
Housing Stability		Not on file	08/05/2023	
Housing Stability		Not on file	08/05/2023	
Interpersonal Safety		Answer	Date Recorded	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Interpersonal Safety		0	08/05/2023	
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Male	01/22/2025 10:41 AM PST	
Legal Sex		Male	05/31/2023 8:08 PM PDT	
Gender Identity		Male	01/22/2025 10:41 AM PST	
Sexual Orientation		Straight	01/22/2025 10:41 AM PST	

Last Filed Vital Signs - documented in this encounter

Not on file

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
Lipid (cholesterol) test	05/04/1983		
COVID-19 Vaccine (#1)	11/04/1983		
DTaP/Tdap/Td Vaccines (1 - Tdap)	05/04/2002		
Hepatitis B Vaccine (1 of 3 - 19+ 3-dose series)	05/04/2002		
Pneumococcal Vaccination (1 of 2 - PCV)	05/04/2002		
Influenza Vaccine (#1)	09/01/2026		
HIB Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
HPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Hepatitis A Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
IPV Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Meningococcal B Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Results - documented in this encounter

Not on file

Document Information

Service Providers	Document Coverage Dates
	Aug. 20, 2014
Custodian Organization	
HOAG MEMORIAL HOSPITAL PRESBYTERIAN One Hoag Drive Newport Beach, CA 92658	
Encounter Providers	Encounter Date
PROVIDER, HOAG IP CONVERSION (Attending, Admitting)	Aug. 20, 2014
Legal Authenticator	
Director H	